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Grade	0.00 out of 100.00



Question 1

Not answered

Marked out of 1.00

Which of the following depot formulations compares favourably with the others listed in terms of symptom control, but produces higher EPSE burden?

Select one:

- ☐ Fluphenazine
- ☐ Flupentixol
- ☐ Haloperidol
- ☐ Zuclopenthixol
- ☐ Pipotiazine

Your answer is incorrect.

Explanation:

Zuclopenthixol depot is as effective as comparator depot typical antipsychotic medications in treating schizophrenia and symptoms of chronic psychosis. However, there is evidence to suggest that it may induce more extrapyramidal side effects such as parkinsonism and dyskinesic movements.

Ref: Ostuzzi G, Bighelli I, So R, Furukawa TA, Barbui C. Does formulation matter? A systematic review and meta-analysis of oral versus long-acting antipsychotic studies. Schizophrenia research. 2017 May 1;183:10-21.

The correct answer is: Zuclopenthixol



Question 2

Not answered

Marked out of 1.00

A male patient on clozapine has gained 20kg in 3 months, though his psychotic symptoms are well controlled. The next step is to

Select one:

- ☐ Reduce the dose of clozapine
- ☐ Initiate lifestyle modifications
- ☐ Switch to high dose haloperidol
- ☐ Switch to risperidone
- ☐ Switch to quetiapine

Your answer is incorrect.

Explanation:

The most effective weight reduction interventions in people with schizophrenia have been shown to be individual lifestyle counselling and exercise interventions. Lifestyle modification should be considered the first line intervention. Following lifestyle counselling and exercise interventions, the next most effective weight reduction interventions have been shown to be psychoeducation, aripiprazole augmentation, topiramate, d-fenfluramine, and metformin.



Management of physical health in schizophrenia

Outcome	Most effective interventions	Effective interventions
Weight reduction	Individual lifestyle counselling Exercise interventions	Psychoeducation Aripiprazole augmentation Topiramate D-fenfluramine Metformin
Waist circumference reduction	Aripiprazole augmentation Topiramate	Dietary interventions
Reducing glucose levels	Switching from olanzapine to quetiapine or aripiprazole Metformin	Glucagon-like peptide-1 receptor agonists Dietary interventions Aripiprazole augmentation
Improvement of insulin resistance	Metformin Rosiglitazone	
Triglycerides and LDL	Topiramate	Lifestyle interventions
Total cholesterol and HDL	Metformin	Lifestyle interventions (for total cholesterol)

Adapted from Vancampfort D et al. 2019



Ref: Vancampfort D et al. The impact of pharmacological and non-pharmacological interventions to improve physical health outcomes in people with schizophrenia: a meta-review of meta-analyses of randomized controlled trials. World Psychiatry. 2019 Feb;18(1):53-66.

The correct answer is: Initiate lifestyle modifications

Question 3

Not answered

Marked out of 1.00

A 45-year-old woman with a long-standing delusional disorder has now developed features of schizophrenia. She is poorly compliant with oral antipsychotics and a community nurse involved in her care suggests trying a depot formulation. Which of the following is true concerning depot antipsychotics?

Select one:

- ☐ Patients consistently do not prefer depot medications, irrespective of their experience
- ☐ The risk of tardive dyskinesia is equal for depot and oral formulations of the same drug
- ☐ The risk of neuroleptic malignant syndrome is higher with depots compared to oral drugs
- ☐ Neuroleptic malignant syndrome is a contraindication for subsequent depot use
- ☐ Atypical antipsychotics are not available in depot formulations

Your answer is incorrect.

Explanation:

The risk of neuroleptic malignant syndrome (NMS) is not higher for depot than oral antipsychotics. There is no evidence to suggest that NMS is a contraindication to subsequent depot use. For the same drug, the risk of tardive dyskinesia is not higher for depot than oral formulations. Some patients on depot like this formulation and many prefer depot to oral medications. Clinicians may wrongly perceive a higher level of stigma being associated with depots which is likely based on the worst characteristics of typical drugs (e.g. unacceptable side-effects) rather than on intramuscular long-acting injections *per se*. There are several atypical antipsychotics available in depot formulations that have been shown to be effective and well-tolerated.

Ref: Carpenter WT, Buchanan RW. Expanding therapy with long-acting antipsychotic medication in patients with schizophrenia. JAMA psychiatry. 2015 Aug 1;72(8):745-6.

The correct answer is: The risk of tardive dyskinesia is equal for depot and oral formulations of the same drug

Question **4**

Not answered

Marked out of 1.00

Which of the following is highly likely to be associated with both hyperglycaemia and weight gain?

Select one:

- ☐ olanzapine
- ☐ haloperidol
- ☐ amisulpride
- ☐ risperidone
- ☐ aripiprazole

Your answer is incorrect.

Explanation:

The strongest evidence for abnormalities of glucose regulation is in people receiving clozapine or olanzapine, with olanzapine also demonstrating greater elevations in glucose levels than other antipsychotics, except clozapine. In the CATIE study, olanzapine caused the most weight gain and the most adverse metabolic effects, including effects on hemoglobin A1C.



Liability for weight gain among oral antipsychotic drugs

Medication	Risk of Weight Gain	Notes
Olanzapine	High	Significantly greater than quetiapine
Clozapine	High	High reported variance of relative weight gain
Chlorpromazine	High/medium	Not significantly differentiated from either medium or high groups
Quetiapine	Medium	Significantly lower than olanzapine and greater than asenapine
Risperidone	Medium	Significantly lower than olanzapine and greater than asenapine
Paliperidone	Medium	Significantly lower than olanzapine and greater than amisulpride
Asenapine	Low	Not significantly greater than haloperidol
Amisulpride	Low	Not significantly greater than haloperidol
Aripiprazole	Low	Not significantly greater than haloperidol
Lurasidone	Low	Not significantly greater than haloperidol or placebo
Ziprasidone	Low	Not significantly greater than haloperidol or placebo
Haloperidol	Low	Not significantly greater than placebo

Adapted from Cooper et al. 2016

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Ref: Cooper SJ et al. BAP guidelines on the management of weight gain, metabolic disturbances and cardiovascular risk associated with psychosis and antipsychotic drug treatment. Journal of Psychopharmacology. 2016 Aug;30(8):717-48.

Swartz MS et al. Special section on implications of CATIE: what CATIE found: results from the schizophrenia trial. Psychiatric services. 2008 May;59(5):500-6.

The correct answer is: olanzapine

Question 5

Not answered

Marked out of 1.00

Nikolas is one of your patients with a primary diagnosis of schizophrenia. His brother is increasingly worried about Nikolas committing suicide. Which of the following drugs has the best evidence for reducing suicidality in schizophrenia compared to the others listed?

Select one:

- ☐ Flupentixol depot
- ☐ Olanzapine
- ☐ Lithium
- ☐ Haloperidol
- ☐ Clozapine

Your answer is incorrect.

Explanation:

Clozapine has the highest quality evidence for reducing suicide risk in schizophrenia, including a prospective RCT, InterSePT (International Suicide Prevention Trial), which showed a 26% reduction in the risk of suicide attempts in clozapine-treated patients compared to those using olanzapine. Further epidemiological studies and meta-analyses have added support to the evidence for clozapine in suicidal patients with schizophrenia.

When comparing the use of first generation antipsychotics (FGAs) versus second generation antipsychotics (SGAs), there is little high quality evidence that the supports the use of FGAs (like haloperidol) for suicide reduction. There is better general data on the use of SGAs for the reduction of suicide, but direct, prospective high quality studies of SGAs are lacking, and there is not any evidence that other antipsychotics possess the anti-suicide properties of clozapine. Given the importance of medication adherence, a long-acting injectable (depot) medication is preferable to no antipsychotic treatment.

There is considerable evidence supporting the efficacy of lithium in reducing suicidal behaviour in patients with bipolar disorder (though there has been some conflicting data). However, there is very little evidence to support its use as being of benefit in helping to reduce suicidal behaviour in patients with schizophrenia.

Ref: Tondo L et al. Prevention of Suicidal Behavior in Bipolar Disorder. Bipolar disorders. 2020 Oct 9.

Turecki G et al. Suicide and suicide risk. Nature reviews Disease primers. 2019 Oct 24;5(1):1-22.

The correct answer is: Clozapine



Question 6

Not answered

Marked out of 1.00

A 22-year-old man presents with anxiety that his genitals are misshapen. He is not convinced, in spite of a plastic surgeon reassuring him by showing him multiple images of normal genital appearances. Which of the following interventions has evidence-based support in this situation?

Select one:

- ☐ Pimozide
- ☐ Lithium
- ☐ Fluoxetine
- ☐ Olanzapine
- ☐ Plastic surgical correction

Your answer is incorrect.

Explanation:

Clinical guidelines recommend SRIs for the pharmacological treatment of body dysmorphic disorder. Most evidence comes from open trials, with only four RCTs conducted to date. RCTs have shown evidence for the efficacy of fluoxetine and escitalopram. To date, there has been one small open trial and one RCT that evaluated pimozide and olanzapine augmentation of fluoxetine, respectively. These studies did not find beneficial effects of augmentation with either medication. Plastic surgery is not advised.

Ref: Krebs G et al. Recent advances in understanding and managing body dysmorphic disorder. Evidence-Based Mental Health. 2017 Aug 1;20(3):71-5.

The correct answer is: Fluoxetine

Question 7

Not answered

Marked out of 1.00

A 34 year old woman has acute onset of intense suspiciousness and passivity phenomenon with auditory hallucinations. With respect to the evidence based treatment of first episode psychosis, which of the following is true?

Select one:

- ☐ There is evidence in favour of lower antipsychotic doses
- ☐ First generation antipsychotics are no better than placebo
- ☐ Choice of an antipsychotic should be based on the gender of the patient
- ☐ Second generation antipsychotics are no better than placebo
- ☐ RCTs show a clearly significant difference between clozapine and other antipsychotics

Your answer is incorrect.

Explanation:

Low to moderate dosage of antipsychotic monotherapy is the first line of treatment for first-episode psychosis. Doses should be well below those for established illness. Approximate, typical, daily mean doses for common antipsychotic medications are as follows: risperidone 2-4mg, haloperidol 2-5mg, quetiapine 400-500mg, amisulpride 450mg, and olanzapine 10mg. Higher doses generally increase the risk of side effects with limited additional therapeutic benefit in first-episode patients.

There is no clear indication from well-designed, double-blind RCTs or well-conducted meta-analyses that any antipsychotic medication is more efficacious than others for first-episode psychosis. Both first and second generation antipsychotics have been shown to be efficacious.

A small number of studies have examined clozapine as a first- or second-line treatment modality in schizophrenia spectrum disorders. Current evidence shows that as a first- or second-line treatment, clozapine outperforms other antipsychotics. When including only RCTs, beneficial effects of clozapine over other antipsychotics become insignificant, although the direction of effect (clozapine outperforming other antipsychotics) remains unchanged.

There is evidence to suggest that women have a better response to medication early in their course of treatment and require lower doses of antipsychotics, though this benefit appears to dissipate with advancing age. Hormonal factors, including changes with child-bearing and menopause, have been proposed but this remains an area of investigation.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. *Journal of Psychopharmacology*. 2020 Jan;34(1):3-78.

Sommer IE et al. The clinical course of schizophrenia in women and men—a nation-wide cohort study. *npj Schizophrenia*. 2020 May 1;6(1):1-7.

Okhuijsen-Pfeifer C et al. Clozapine as a first-or second-line treatment in schizophrenia: a systematic review and meta-analysis. *Acta Psychiatrica Scandinavica*. 2018 Oct;138(4):281-8.

The correct answer is: There is evidence in favour of lower antipsychotic doses



Question 8

Not answered

Marked out of 1.00

A patient with unremitting continuous schizophrenic illness is considered for clozapine therapy. Before initiating clozapine, all of the following must be excluded as possible causes of the patient's poor treatment response except

Select one:

- ☐ Nonadherence with medications
- ☐ High expressed emotions in the family
- ☐ Co-morbid substance use
- ☐ Presence of passivity symptoms
- ☐ Dosage and duration of previous treatments

Your answer is incorrect.

Explanation:

Clozapine is the only drug licensed for treatment-resistant schizophrenia in the UK. It should be the first choice of treatment for patients whose illness has shown a poor response to sequential trials of two (or more) different antipsychotic medications that have been adequate in terms of dosage and duration. Poor medication adherence and comorbid substance use should be excluded as causes of the apparent poor response to antipsychotic medication. Course of illness and medication adherence can be influenced by a variety of factors, including attributes and attitudes of caregivers and family members. Psychosocial factors should be assessed and family involvement should be incorporated, as appropriate, into a comprehensive treatment plan.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: Presence of passivity symptoms

Question 9

Not answered

Marked out of 1.00

Which of the following has been tried as an add-on therapy with antipsychotics for negative symptoms in patients with deficit syndrome of schizophrenia?

Select one:

- ☐ Acamprosate
- ☐ Nortriptyline
- ☐ Ketamine
- ☐ Fish oils
- ☐ D-cycloserine

Your answer is incorrect.

Explanation:

Dysregulation of N-methyl D-aspartate (NMDA) receptor signaling has been implicated in schizophrenia. Therapeutic approaches to maintain a sustained increase in agonist activity at the glycine site of the NMDA receptor have produced some promising, though inconsistent, efficacy for negative symptoms. The glycine site partial agonist, **D-cycloserine** (DCS) has been shown to improve negative symptoms at a daily dose of 50mg in patients treated with first generation antipsychotics and at a dose of 100mg/d in medication-free patients. In other trials, including the multicenter CONSIST trial, neither DCS nor glycine (a full agonist) improved negative symptoms when added to predominantly second generation antipsychotics.

A single infusion of ketamine has been shown to produce social withdrawal, attenuated psychotic symptoms and memory impairment in healthy subjects and a transient relapse of psychosis in schizophrenia patients stabilised on haloperidol and in those who are neuroleptic free.

Several different medications have been tried in combination with antipsychotics for the treatment of negative symptoms of schizophrenia, including SSRIs and specifically, fluoxetine.

Ref: C Goff D. D-cycloserine in schizophrenia: new strategies for improving clinical outcomes by enhancing plasticity. *Current Neuropsychopharmacology*. 2017 Jan 1;15(1):21-34.

Goff DC et al. A placebo-controlled trial of fluoxetine added to neuroleptic in patients with schizophrenia. *Psychopharmacology*. 1995 Feb 1;117(4):417-23.

Correll CU et al. Efficacy of 42 pharmacologic cotreatment strategies added to antipsychotic monotherapy in schizophrenia: systematic overview and quality appraisal of the meta-analytic evidence. *JAMA psychiatry*. 2017 Jul 1;74(7):675-84.

The correct answer is: D-cycloserine



Question 10

Not answered

Marked out of 1.00

A 65-year-old woman with history of schizophrenia is not taking her medication. She is currently psychotic. When her son reminds her to take her medications they end up having arguments. Which of the following medications is indicated?

Select one:

- ☐ Olanzapine
- ☐ Quetiapine
- ☐ Amisulpride
- ☐ Antipsychotic depot injection
- ☐ Clozapine

Your answer is incorrect.

Explanation:

BAP guidelines (2020) clearly state that a depot/long-acting injectable (LAI) antipsychotic formulation should be considered when this is preferred by the patient, previous non-adherence has led to frequent relapse, or the avoidance of non-adherence is a clinical priority. LAIs make adherence transparent and reduce the practical difficulties associated with taking daily medication. The decision to use a depot should be made on an individual basis and the patient fully involved in the decision-making process.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: Antipsychotic depot injection



Question 11

Not answered

Marked out of 1.00

Which of the following first generation antipsychotics was used in CATIE study?

Select one:

- ☐ Pipotiazine
- ☐ Haloperidol
- ☐ Perphenazine
- ☐ Promethiazine
- ☐ Prochlorperazine

Your answer is incorrect.

Explanation:

CATIE stands for Clinical Antipsychotic Trials of Intervention Effectiveness. The study design was a double-blind pragmatic RCT. In total, 1493 patients with chronic schizophrenia (mean duration of illness = 14 years) were enrolled across 57 sites from 2001 to 2004. Olanzapine, quetiapine, risperidone, ziprasidone (added later in the trial) and perphenazine (the only typical drug) were used in phase 1, and clozapine was added in phase 2.

Ref: Swartz MS et al. Special section on implications of CATIE: what CATIE found: results from the schizophrenia trial. Psychiatric services. 2008 May;59(5):500-6.

The correct answer is: Perphenazine

Question 12

Not answered

Marked out of 1.00

Current clinical guidelines for management of schizophrenia support all of the following practices except

Select one:

- ☐ Depot medication can be used to help with adherence
- ☐ Combined antipsychotic prescriptions must be tried only in first episode
- ☐ No routine switch is needed from typicals to atypicals
- ☐ Treatment choice must be based on information sharing between patient and clinician
- ☐ Consider clozapine early, following non-response to 2 different antipsychotic trials

Your answer is incorrect.

Explanation:

Recommendations from NICE guidelines (2014):

- Choice of antipsychotic medication should be made by the patient and healthcare professional together, with the provision of information and discussion of the likely benefits and possible side effects of each drug
- Offer clozapine to people with schizophrenia whose illness has not responded adequately to treatment despite the sequential use of adequate doses of at least 2 different antipsychotic drugs (at least one should be a non-clozapine second-generation antipsychotic)
- Do not initiate regular combined antipsychotic medication, except for short periods (e.g. when changing medications)
- Consider offering depot/LAI antipsychotic medication to those who would prefer such treatment and when avoiding covert non-adherence (either intentional or unintentional) to antipsychotic medication is a clinical priority (BAP 2020 similarly states that depot/LAI antipsychotic formulations should be considered when the need to monitor medication adherence is a priority or if the patient prefers this approach)

With respect to FGAs vs. SGAs, BAP 2020 notes that there is no apparent disadvantage in choosing to prescribe an FGA rather than SGA in terms of quality of life, symptoms, or the associated costs of care. Choice of a maintenance antipsychotic medication for an individual patient is more a question of correct dose and formulation than drug group.

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of psychosis and schizophrenia in adults. Updated edition 2014.

The correct answer is: Combined antipsychotic prescriptions must be tried only in first episode



Question 13

Not answered

Marked out of 1.00

A 44 year old man admitted to the secure intensive care unit shows a persistently high degree of hostility and aggressiveness. He was diagnosed with schizophrenia 20 years ago. Which of the following medications has the best evidence in reducing persistent hostility and aggression in schizophrenia compared to the others listed?

Select one:

- ☐ Clozapine
- ☐ Haloperidol
- ☐ Olanzapine
- ☐ Zuclopenthixol acetate
- ☐ Chlordiazepoxide

Your answer is incorrect.

Explanation:

BAP guidelines (2020) outline the following for the pharmacological management of persistent hostility/aggression in schizophrenia:

- Although in the CATIE study, Olanzapine was found to have advantages in terms of a specific anti-hostility effect over the other antipsychotics tested, the antipsychotic with the best evidence of an anti-hostility effect in schizophrenia is clozapine
- Reduction of hostility and aggression with clozapine found in RCTs has appeared to be independent of improvement in positive symptoms, or side effects such as akathisia or sedation
- A trial of clozapine should be considered for people with schizophrenia characterised by persistent symptoms of aggression and hostility

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: Clozapine

Question 14

Not answered

Marked out of 1.00

The following scenarios depict five different patients with schizophrenia. Which one of the following depictions is associated with a comparatively positive prognostic outlook for schizophrenia?

Select one:

- ☐ Mr B had an acute onset psychosis at the age of 25. His family blames him for his illness. He stays at a care home where the staff do not let him be with his family for more than 24 hours a week.
- ☐ Mr A has predominantly negative symptoms and prefers to stay alone, avoiding his friends who often urge him to go for walking trips.
- ☐ Mr C's major residual symptom is disorganised speech. On neurological examination he shows grasp reflex and dysdiadochokinesia.
- ☐ Mr D had a very gradual and quiet onset of symptoms when he was 16 years of age.
- ☐ Mrs C is a 52-year-old woman who developed schizophrenia after getting married. Her husband deserted her following the diagnosis.

Your answer is incorrect.

Social isolation, living alone, having soft neurological signs and young onset are poor prognostic factors in schizophrenia. 'Expressed emotion' was a concept originally developed by Brown and Rutter in 1966 as a part of the Camberwell Family Interview (audio-taped interview with carer), and later modified by Vaughn and Leff in 1976. The ratings are based on content and prosodic aspects and emphasis of speech. Five measures are considered; 1. critical comments 2. positive remarks 3. emotional overinvolvement 4. hostility 5. emotional warmth. A large amount of face-to-face contact (more than 35 hours per week) with a relative with a high expressed emotion score increased the risk of relapse, but in households with a low expressed emotion score, high levels of contact appeared to be protective. Mr B is somewhat protected as his contact is kept minimal by the care home staff.

The correct answer is: Mr B had an acute onset psychosis at the age of 25. His family blames him for his illness. He stays at a care home where the staff do not let him be with his family for more than 24 hours a week.

Question 15

Not answered

Marked out of 1.00

Which of the following psychosocial interventions has an established evidence base in the management of schizophrenia?

Select one:

- ☐ Vocational rehabilitation
- ☐ Cognitive behavioural therapy
- ☐ Family therapy
- ☐ Psychoeducation
- ☐ All of the listed options

Your answer is incorrect.

Explanation:

As reflected by the most recent NICE guidelines (2014), CBT, family intervention, psychoeducation, and vocational/employment intervention: all have an established evidence base in the management of schizophrenia.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of psychosis and schizophrenia in adults. Updated edition 2014.

The correct answer is: All of the listed options

Question **16**

Not answered

Marked out of 1.00

Which of the following obstetric events is associated with an increased risk of schizophrenia?

Select one:

- ☐ Foetal hypoxia at delivery
- ☐ Premature rupture of membranes
- ☐ Low birth weight
- ☐ Pregnancy induced hypertension
- ☐ All of the listed options

Your answer is incorrect.

Explanation:



Obstetrical complications as risk factors for schizophrenia

Complications of pregnancy	Fetal abnormalities of growth and development	Complications of delivery
• Bleeding • Pregnancy-induced hypertension • Pre-eclampsia (high blood pressure, proteinuria, oedema) • Diabetes • Rhesus incompatibility • Placental abruption • Premature rupture of membranes	• Low birth weight • Prematurity • Congenital malformations • Small head circumference	• Hypoxia/asphyxia • Uterine atony • Use of forceps during delivery • Emergency Caesarean section • Resuscitation • Use of an incubator

Adapted from Meli et al. 2012

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Ref: Meli G et al. Prenatal and perinatal risk factors of schizophrenia. The Journal of Maternal-Fetal & Neonatal Medicine. 2012 Dec 1;25(12):2559-63.

The correct answer is: All of the listed options

Question 17

Not answered

Marked out of 1.00

Which of the following was considered a characteristic feature of 'simple schizophrenia'?

Select one:

- ☐ Social withdrawal
- ☐ Memory loss
- ☐ Delusions
- ☐ Motor incoordination
- ☐ Auditory hallucinations

Your answer is incorrect.

Explanation:

Simple schizophrenia was an uncommon subtype of schizophrenia first characterised by Eugene Bleuler in 1911. An insidious onset of prominent negative symptoms and lack of delusions, hallucinations, and thought disorder were considered the essential clinical features. These features were thought to contribute to its chronic course, with significant social withdrawal and deterioration in occupational functioning compared with other schizophrenia subtypes.

Ref: Lally J et al. Simple schizophrenia: a forgotten diagnosis in psychiatry. The Journal of nervous and mental disease. 2019 Sep 1;207(9):721-5.

The correct answer is: Social withdrawal

Question 18

Not answered

Marked out of 1.00

The single most important factor that increases the risk of hospitalisation in the mentally ill is

Select one:

- ☐ Being female
- ☐ Having co-morbidities
- ☐ Treatment non-adherence
- ☐ Being elderly
- ☐ Being an adolescent

Your answer is incorrect.

Explanation:

Patients with major psychiatric disorders are most likely to be non-adherent to medication due to poor reasoning and lack of insight about their illness and treatment. Psychotropic medication non-adherence can lead to illness exacerbation, reduce treatment effectiveness, and leave patients less responsive to subsequent treatment. Other consequences of non-adherence include hospitalisation/re-hospitalisation, poor quality of life or psychosocial outcomes, relapse of symptoms, increased comorbid medical conditions, wastage of health care resources, and increased suicide.

In addition to non-adherence, risk factors known to predict hospitalisation and/or re-hospitalisation include younger age, male gender, drug formulation (e.g. oral vs. long-acting injectable), medication nonresponse, substance abuse, longer duration of hospitalisation, prior hospitalisation, multiple and more recent (vs. distant) hospitalisations, being discharged against medical advice, and living in a supervised setting.

Ref: Semahegn A et al. Psychotropic medication non-adherence and its associated factors among patients with major psychiatric disorders: a systematic review and meta-analysis. Systematic reviews. 2020 Dec;9(1):1-8.

Yan T et al. Hospitalization risk factors in antipsychotic-treated schizophrenia, bipolar I disorder or major depressive disorder. Journal of comparative effectiveness research. 2019 Mar;8(4):217-27.

The correct answer is: Treatment non-adherence



Question 19

Not answered

Marked out of 1.00

In the PANSS scale for schizophrenia, which of the following is coded as a positive symptom?

Select one:

- ☐ Lack of insight
- ☐ Disturbance of volition
- ☐ Unusual thought content
- ☐ Mannerisms
- ☐ Conceptual disorganisation

Your answer is incorrect.

Explanation:

Kay and colleagues developed the Positive and Negative Syndrome Scale (PANSS) which quantifies positive, negative, and more general symptom facets (i.e. cognitive, mood, motor, and thought process abnormality symptoms) of schizophrenia. It consists of 30 items, with each item rated on a seven-point severity scale. Symptoms are categorised into three dimensions: positive, negative, and general psychopathology.



Positive and Negative Syndrome Scale (PANSS)

Positive subscale	Negative subscale	General psychopathology subscale
• Delusions • Conceptual disorganisation • Hallucinations • Excitement • Grandiosity • Suspiciousness/persecution • Hostility	• Blunted affect • Emotional withdrawal • Poor rapport • Passive/apathetic social withdrawal • Difficulty in abstract thinking • Lack of spontaneity and flow of conversation • Stereotyped thinking	• Somatic concern • Anxiety • Guilt feelings • Tension • Mannerisms and posturing • Depression • Motor retardation • Uncooperativeness • Unusual thought content • Disorientation • Poor attention • Lack of judgement and insight • Disturbance of volition • Poor impulse control • Preoccupation • Active social avoidance

Kay, Opler, and Fiszbein 1987

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Ref: Lefort-Besnard J et al. Patterns of schizophrenia symptoms: hidden structure in the PANSS questionnaire. Translational psychiatry. 2018 Oct 30;8(1):1-0.

The correct answer is: Conceptual disorganisation

Question 20

Not answered

Marked out of 1.00

What is the risk of schizophrenia in a child if both parents have schizophrenia?

Select one:

- ☐ 5-10%
- ☐ 20-30%
- ☐ 50-60%
- ☐ 40-50%
- ☐ 10-20%

Your answer is incorrect.

Explanation:

The likelihood of a person having schizophrenia is correlated with the closeness of the relationship to an affected relative. Having a first-degree relative (e.g. one parent or one sibling) with schizophrenia increases a person's risk to 12-15%. The risk jumps to 40% if both parents have schizophrenia. In the case of monozygotic twins who have identical genetic endowment, there is an approximately 46% concordance rate for schizophrenia.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 360.

Sample D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 191.

The correct answer is: 40-50%

Question **21**

Not answered

Marked out of 1.00

Which of the following statements best summarises the available evidence for managing cognitive symptoms associated with schizophrenia?

Select one:

- ☐ No conclusive evidence exists for the use of a specific agent in the management of cognitive symptoms
- ☐ Cognitive symptoms occur only in association with negative symptoms
- ☐ Cognitive symptoms respond best to anti-dementia drugs
- ☐ Amisulpride has the best evidence for managing cognitive symptoms in the absence of negative symptoms
- ☐ RCTs with paired observations clearly demonstrate cognitive improvement with atypicals

Your answer is incorrect.

Explanation:

As highlighted in the most recent BAP guidelines (2020):

- Cognitive dysfunctions, which can be differentiated from negative symptoms, may be more predictive of functional outcome in schizophrenia than either negative or positive symptoms
- The evidence for use of anti-dementia drugs such as acetylcholinesterase inhibitors and memantine is too weak at present to recommend their use in schizophrenia
- While there may be hidden improvements in specific cognitive domains in subgroups of patients treated with antipsychotic medications with differing pharmacological profiles, this has yet to be clearly demonstrated
- The best current pharmacological strategy for managing cognitive impairment in schizophrenia is to ensure, for each patient, that the lowest effective dose of antipsychotic medication is used; no conclusive evidence exists for use of a specific agent

Ref: Barnes TR et al. Evidence-based guidelines for the pharmacological treatment of schizophrenia: Updated recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2020 Jan;34(1):3-78.

The correct answer is: No conclusive evidence exists for the use of a specific agent in the management of cognitive symptoms

Question **22**

Not answered

Marked out of 1.00

SSRI-induced sexual dysfunction occurs secondary to

Select one:

- ☐ Histaminergic blockade
- ☐ 5HT3 blockade
- ☐ 5HT1 stimulation
- ☐ 5HT2 blockade
- ☐ 5HT2 stimulation

Your answer is incorrect.

Explanation:

SSRIs inhibit 5HT reuptake via antagonism of the serotonin transporter which increased synaptic 5HT. Although the underlying mechanisms are not completely elucidated, it is generally accepted that 5HT serves to diminish sexual function and dopamine enhances sexual function. It is thought that stimulation of 5HT_{2A} receptors has negative effects on sexual function. Antidepressant drugs that increase 5HT signaling (e.g. SSRIs) in the absence of 5HT_{2A} antagonism are commonly associated with sexual side effects while agents which possess 5HT_{2A} antagonist activity (e.g. mirtazapine) have a relatively low likelihood of inducing sexual side effects.

Ref: Osis L, Bishop JR. Pharmacogenetics of SSRIs and sexual dysfunction. *Pharmaceuticals*. 2010 Dec;3(12):3614-28.

The correct answer is: 5HT2 stimulation

Question **23**

Not answered

Marked out of 1.00

Which of the following strategies has better efficacy with respect to the administration of ECT?

Select one:

- ☐ Using sine wave instead of brief pulse ECT
- ☐ Using a higher stimulus dose instead of a lower stimulus dose
- ☐ Using unilateral instead of bilateral ECT
- ☐ Using thrice weekly instead of twice weekly ECT
- ☐ None of the listed options

Your answer is incorrect.

Explanation:

The Royal College of Psychiatrists' statement on ECT (2017) notes the following:

- Bilateral ECT is more effective than unilateral ECT but may cause more cognitive impairment
- With unilateral ECT, a higher stimulus dose is associated with greater efficacy, but also increased cognitive impairment compared with a lower stimulus dose
- Twice weekly ECT offers the best balance between therapeutic outcomes and adverse effects (a comprehensive review and meta-analysis by Charlson et al noted that twice weekly ECT is associated with a similar efficacy to thrice weekly ECT, and there is significantly greater efficacy for thrice weekly ECT compared to once weekly ECT)

Also of note: as a result of cognitive side effects associated with sine wave current, brief-pulse wave is considered standard treatment. The use of sine-wave constant voltage and constant energy devices is not considered justified.

Ref: Royal College of Psychiatrists. Statement on electroconvulsive therapy (ECT). 2017

Leiknes KA et al. Contemporary use and practice of electroconvulsive therapy worldwide. Brain and behavior. 2012 May;2(3):283-344.

Charlson F et al. ECT efficacy and treatment course: a systematic review and meta-analysis of twice vs thrice weekly schedules. Journal of affective disorders. 2012 Apr 1;138(1-2):1-8.

The correct answer is: Using a higher stimulus dose instead of a lower stimulus dose



Question **24**

Not answered

Marked out of 1.00

A community health worker regularly sees a patient with OCD. She wants to use a scale to screen for depression before asking the patient to attend your clinic for an evaluation. Which scale would you choose?

Select one:

- ☐ Montgomery Asberg Rating Scale
- ☐ Edinburgh Depression Scale
- ☐ Schedule for Clinical Assessment in Neuropsychiatry
- ☐ Beck Depression Inventory
- ☐ Calgary Depression Scale

Your answer is incorrect.

Explanation:

The Beck Depression Inventory is a 21-item patient-rated assessment scale for depression that was developed by Aaron Beck and colleagues in 1961. There have been multiple subsequent revisions. The easy applicability and psychometric soundness of this scale have popularised its use in a variety of settings, including the community.

Clinician-administered instruments such as the Montgomery-Asberg Depression Rating Scale and the Schedules for Clinical Assessment in Neuropsychiatry are onerous to implement in the community setting and not appropriate for self-screening.

The Calgary Depression Scale for Schizophrenia is a 9-item clinician-rated outcome measure that assesses level of depression in people with schizophrenia.

The Edinburgh Postnatal Depression Scale is a 10-item tool developed for screening postpartum women for depression in outpatient clinics, home visits, or during postpartum appointments.

Ref: Wang YP, Gorenstein C. Assessment of depression in medical patients: a systematic review of the utility of the Beck Depression Inventory-II. Clinics. 2013;68(9):1274-87.

Smith-Nielsen J et al. Validation of the Edinburgh Postnatal Depression Scale against both DSM-5 and ICD-10 diagnostic criteria for depression. BMC psychiatry. 2018 Dec;18(1):1-2.

The correct answer is: Beck Depression Inventory

Question **25**

Not answered

Marked out of 1.00

The highest risk of serotonin syndrome is seen in which of the following patients?

Select one:

- ☐ 44 year old taking trazadone and fluoxetine
- ☐ 55 year old taking phenelzine and escitalopram
- ☐ 67 year old taking amitriptyline and phenelzine
- ☐ 32 year old taking citalopram
- ☐ 44 year old taking venlafaxine and mirtazapine

Your answer is incorrect.

Explanation:

Serotonin syndrome can occur with a single serotonergic drug at a therapeutic dose or more frequently in combination of serotonergic drugs or in overdose. Most severe cases of serotonin syndrome involve an MAOI (e.g. phenelzine; also including moclobemide) plus an SSRI (e.g. escitalopram).

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 337.

The correct answer is: 55 year old taking phenelzine and escitalopram

Question **26**

Not answered

Marked out of 1.00

You are treating 5 patients with depression in your outpatient unit. Which of the following patients is highly likely to drop out of treatment?

Select one:

- ☐ 26 year old who is a school drop-out
- ☐ 25 year old man with 3 previous depressive episodes
- ☐ 72 year old retired clerk
- ☐ 44 year old school teacher
- ☐ 33 year old with more subjective than objective depression

Your answer is incorrect.

Explanation:

Attrition occurs when patients discontinue acute treatment prematurely. In RCTs, attrition during treatment for depression has been shown to be predicted by younger age, racial or ethnic minority status, male gender, unemployment, fewer years of education, poorer social adjustment, lower income, greater depressive symptom severity, and greater anxiety. In the STAR*D (Sequenced Treatment Alternatives to Relieve Depression) study – a large, real-world clinical trial – immediate attrition was associated with younger age, less education, and higher perceived mental health functioning. Attrition later in treatment was associated with younger age, less education, and African American race. Experience with more than one episode of depression was associated with less attrition.

Ref: Warden D et al. Predictors of attrition during initial (citalopram) treatment for depression: a STAR* D report. American Journal of Psychiatry. 2007 Aug;164(8):1189-97.

The correct answer is: 26 year old who is a school drop-out

Question 27

Not answered

Marked out of 1.00

A GP writes to you asking for clarification regarding depression treatment guidelines. Which of the following practices, if employed, is not recommended?

Select one:

- ☐ He uses SSRIs as first line drugs
- ☐ In cases of co-morbid anxiety and depression, he treats anxiety first
- ☐ In mild depression, if the patient agrees, he provides information and offers follow-up in 2 weeks
- ☐ He undertakes a comprehensive assessment that does not rely on symptom count
- ☐ He does not give antidepressants to those with mild depression at first consultation

Your answer is incorrect.

Explanation:

According to NICE guidelines (updated 2019), when depression is accompanied by anxiety symptoms, the first priority is to treat depression. When assessing a person who may have depression, a comprehensive assessment that does not rely simply on a symptom count should be conducted. For people who may recover without intervention, or have mild depression and do not want intervention, it is recommended to discuss concerns, provide information, and arrange follow-up in 2 weeks. It is not recommended to use antidepressants routinely to treat persistent subthreshold depressive symptoms or mild-moderate depression because the risk-benefit ratio is poor. In cases where pharmacological treatment is indicated, an SSRI is usually considered first-line.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment and management of depression in adults. Updated edition 2019.

The correct answer is: In cases of co-morbid anxiety and depression, he treats anxiety first

Question 28

Not answered

Marked out of 1.00

A patient being treated with antidepressants loses faith in the treatment and asks you if antidepressants really work. Which of the following is an accurate response?

Select one:

- ☐ They work if one takes them for at least 4 months
- ☐ They work in moderate depression, though with increasing severity they lose efficacy
- ☐ They work well for adults, though not so well for the elderly
- ☐ They work best for young children as they take them regularly
- ☐ One additional response occurs for every 5 depressed adults who take antidepressants

Your answer is incorrect.

Explanation:

Antidepressants are effective in the acute treatment of major depression of moderate and greater severity in adults (response rates of about 48-50% compared with 30-32% on placebo, NNT = 5-7). Evidence is conflicting about whether the benefits of antidepressants increase with increasing severity in moderate to severe depression. Factors most consistently associated with efficacy of antidepressants are:

- 1) major depression that is clearly above the threshold for diagnosis in both number and severity of individual symptoms AND
- 2) a duration of at least 2-3 months

The response rate for antidepressant treatment of late life depression is comparable to younger adults. The number needed to treat for major depression in the elderly is 5-7 (similar to other age groups). While the elderly may be a little slower to respond than younger adults, rate differences are not clinically significant. Smaller drug-placebo differences (due to greater responses to placebo) are seen in children and adolescents versus adults. In children <13 years, the drug-placebo difference is small and not statistically significant. Children 5-11 years old should only be offered fluoxetine when psychological therapy has been ineffective.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the identification and management of depression in children and young people. 2019.

Cleare A et al. Evidence-based guidelines for treating depressive disorders with antidepressants: a revision of the 2008 British Association for Psychopharmacology guidelines. *Journal of Psychopharmacology*. 2015 May;29(5):459-525.

The correct answer is: One additional response occurs for every 5 depressed adults who take antidepressants



Question 29

Not answered

Marked out of 1.00

A primary care physician is concerned about prescribing an antidepressant medication to his 35-year-old patient because he has read that antidepressants can lead to suicide. Which of the following is the MOST CORRECT statement regarding the association of antidepressants with suicide?

Select one:

- ☐ Antidepressant (including SSRI) treatment is associated with increased risk of completed suicide
- ☐ Suicides in depressed patients occur more often among those who are not taking an antidepressant
- ☐ Treatment-emergent suicidal ideation tends to disappear progressively after the first 4-6 months of treatment
- ☐ Starting doses of antidepressants have no impact on the risk of suicidal ideation or attempts
- ☐ The risk of new-onset suicidal ideation with antidepressants is highest in the elderly

Your answer is incorrect.

Explanation:

Antidepressant (including SSRI) treatment is not associated with increased risk of completed suicide. Suicides in depressed patients occur more often among those who are not taking an antidepressant. Treatment-emergent suicidal ideation is infrequent in adults; when it occurs, it tends to disappear progressively in the first 4-6 weeks of treatment. Starting treatment with high doses of antidepressants (beyond the recommendations) seems to increase the risk of suicidal ideation or attempts. The risk of new suicidal ideation is largely confined to <25 yr olds (probably influenced by more frequent substance abuse and impulsive aggression in youth).

Ref: Courtet P, Lopez-Castroman J. Antidepressants and suicide risk in depression. World Psychiatry. 2017 Oct;16(3):317.

Stübner S et al. Suicidal ideation and suicidal behavior as rare adverse events of antidepressant medication: current report from the AMSP multicenter drug safety surveillance project. International journal of neuropsychopharmacology. 2018 Sep;21(9):814-21.

The correct answer is: Suicides in depressed patients occur more often among those who are not taking an antidepressant

Question **30**

Not answered

Marked out of 1.00

A patient living in the rural Scottish highlands has an episode of depression. Despite her husband's insistence she refuses any intervention or treatment. It is most likely that her depressive episode will last

Select one:

- ☐ 3 to 4 weeks
- ☐ 1 to 3 years
- ☐ 3 to 4 months
- ☐ 6 to 12 months
- ☐ 6 to 12 weeks

Your answer is incorrect.

Explanation:

An untreated depressive episode typically lasts 6 to 13 months. Most treated episodes last about 3 months. As the disorder progresses, patients tend to have more frequent episodes that last longer.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 360. p. 387.

The correct answer is: 6 to 12 months



Question **31**

Not answered

Marked out of 1.00

The median duration of a treated episode of depression is

Select one:

- ☐ 6 months
- ☐ 6 weeks
- ☐ 12 weeks
- ☐ 4 weeks
- ☐ 12 months

Your answer is incorrect.

Explanation:

An untreated depressive episode typically lasts 6 to 13 months. Most **treated** episodes last about 3 months. As the disorder progresses, patients tend to have more frequent episodes that last longer.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 360. p. 387.

The correct answer is: 12 weeks



Question 32

Not answered

Marked out of 1.00

A depressed patient is a keen naturalist. He wants to try a herbal remedy for depression. What would you choose from the following options?

Select one:

- ☐ Panax ginseng
- ☐ Valeriana officinalis
- ☐ Piper methysticum
- ☐ Gingko biloba
- ☐ Hypericum perforatum

Your answer is incorrect.

Explanation:

BAP guidelines (2015) indicate the following about extracts of **St John's Wort** (*Hypericum perforatum*):

- **Effective** in the acute treatment of **mild-moderate major depression**; appear comparable in efficacy to antidepressants and well tolerated
- **Apparent efficacy in milder depression** likely reflects methodological problems with older trials
- **Longer-term efficacy and safety are not established**
- Potential to **interact adversely** with other medications including **antidepressants**
- **Not recommended as a first-line treatment for depression** (given only preliminary medium-term data and lack of longer-term and relapse prevention data)
- Could be **considered** for **mild and moderate depression** where **first-line treatments are not possible or not tolerated**, provided a **recognised standardized preparation** is used

Ref: Cleare A et al. Evidence-based guidelines for treating depressive disorders with antidepressants: a revision of the 2008 British Association for Psychopharmacology guidelines. Journal of Psychopharmacology. 2015 May;29(5):459-525.

The correct answer is: Hypericum perforatum



Question 33

Not answered

Marked out of 1.00

A psychiatrist diagnoses a depressive disorder using ICD-11 criteria in a 24-year-old lady who attends her outpatient clinic. Which of the following diagnosis is she most likely to have if subsequently followed up for several years?

Select one:

- ☐ Generalised anxiety disorder
- ☐ Alcohol use disorder
- ☐ Bipolar disorder
- ☐ Depressive disorder
- ☐ Schizophrenia

Your answer is incorrect.

Explanation:

Diagnostic stability is a pertinent concern in psychiatry. In the case of unipolar depression, the current diagnostic classifications have managed to achieve considerably reliability. Depressive disorders follow a chronic course in at least half of all cases, with symptom patterns that usually remain stable over time. According to an 11-year follow-up study of adults diagnosed with major depressive disorder (MDD), dysthymia, or both, 34% of those with MDD and 43% with dysthymia had their diagnoses switched to other forms of depression, anxiety, or alcohol use disorder.

Ref: de la Vega D et al. A review on the general stability of mood disorder diagnoses along the lifetime. Current Psychiatry Reports. 2018 Apr;20(4):1-0.

The correct answer is: Depressive disorder

Question **34**

Not answered

Marked out of 1.00

Which of the following anti-epileptics causes visual field defects?

Select one:

- ☐ Sodium valproate
- ☐ Topiramate
- ☐ Lamotrigine
- ☐ Vigabatrin
- ☐ Gabapentin

Your answer is incorrect.

Explanation:

Visual disturbances are a common side effect of many antiepileptic drugs (AED). Non-specific visual abnormalities, that are often associated with over-dosage and prolonged AED use, include diplopia, blurred vision, and nystagmus. Some AEDs are associated with specific visual problems that can occur even when the drugs are administered within the recommended daily dose. Vigabatrin has been associated with bilateral concentric visual field loss and central visual function deficits.

Visual side effects of antiepileptic drugs

Carbamazepine	Blurred vision, diplopia, abnormal colour perception, nystagmus, oscillopsia, photosensitivity, altered visual evoked potentials (VEPs)
Gabapentin	Blurred vision, diplopia, nystagmus, some visual electrophysiological changes
Lamotrigine	Blurred vision, diplopia, nystagmus, some visual electrophysiological disturbances
Phenytoin	Blurred vision, disturbed colour perception, nystagmus, ophthalmoplegia, dyskinesia
Sodium valproate	Abnormal colour perception, altered visual evoked potentials (VEPs)
Topiramate	Diplopia, acute myopia and angle closure glaucoma
Vigabatrin	Diplopia, colour perception abnormalities, nystagmus, peripheral visual field loss, retinal abnormalities, optic nerve pallor, visual electrophysiological changes, reduced contrast sensitivity, reduced ocular blood flow

Adapted from Roff Hilton et al. 2004

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Ref: Roff Hilton EJ, Hosking SL, Betts TI. The effect of antiepileptic drugs on visual performance. Seizure. 2004 Mar 1;13(2):113-28.

The correct answer is: Vigabatrin

Question 35

Not answered

Marked out of 1.00

A known bipolar patient has had many episodes of mania but no clinically significant depressive symptoms in the past. Currently, he presents with severe depression. He is already on lithium with good serum levels and is compliant. Of the listed options, what should be the next step?

Select one:

- ☐ Add lamotrigine
- ☐ No intervention required
- ☐ Add venlafaxine
- ☐ Increase lithium dose
- ☐ Add sertraline

Your answer is incorrect.

Explanation:

As reflected in the most recent NICE, BAP, and CANMAT/ISBD bipolar treatment guidelines, there are several options in this scenario, including the addition of olanzapine+fluoxetine, quetiapine, olanzapine, lamotrigine, or lurasidone.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

Goodwin GM et al. Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2016 Jun;30(6):495-553.

The correct answer is: Add lamotrigine



Question **36**

Not answered

Marked out of 1.00

A 45-year-old woman has recurrent depression, as well as hypomanic episodes that occur only when she is treated with ECT. She has had 4 episodes of ECT-induced hypomania in the past. According to Akiskal, which of the following terms may be used to describe her illness?

Select one:

- ☐ Bipolar I
- ☐ Bipolar III
- ☐ Bipolar IV
- ☐ Bipolar II 1/2
- ☐ Bipolar II

Your answer is incorrect.

Explanation:

In Akiskal's schema of bipolar spectrum, bipolar III is described as depression with induced hypomania (i.e., hypomania occurring solely in association with antidepressant or other somatic treatment).

Ref: Tavormina G. Bipolar disorders and bipolarity: the notion of the "mixture". *Psychiatria Danubina*. 2019 Sep 4;31(suppl 3):434-7.

Akiskal HS, Pinto O. The evolving bipolar spectrum: prototypes I, II, III, and IV. *Psychiatric Clinics of North America*. 1999 Sep 1;22(3):517-34.

The correct answer is: Bipolar III



Question 37

Not answered

Marked out of 1.00

Which of the following has RCT evidence in favour of treatment during acute episodes of mania?

Select one:

- ☐ Bupropion
- ☐ Selegiline
- ☐ Tiagabine
- ☐ Clonazepam
- ☐ Diazepam

Your answer is incorrect.

Explanation:

Small RCTs have shown some efficacy for clonazepam in the treatment of acute mania. Clinical practice guidelines such as BAP (2016) recognise the usefulness of benzodiazepines such as diazepam, lorazepam, or clonazepam in the management of acutely agitated manic states, but recommend their use only as adjunctive treatment:

"To promote sleep in agitated overactive patients in the short term, consider adjunctive treatment with GABA modulating medications (benzodiazepines)."

Ref: Goodwin GM et al. Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2016 Jun;30(6):495-553.

The correct answer is: Clonazepam

Question 38

Not answered

Marked out of 1.00

A 44 year old patient has been admitted 7 times with relapses of bipolar disorder in last few years. Which of the following investigations is very important in this patient?

Select one:

- ☐ Full blood count
- ☐ Liver function, especially GGT
- ☐ Thyroid function test
- ☐ Folate levels
- ☐ MRI brain

Your answer is incorrect.

Explanation:

Rapid cyclers have higher rates of hypothyroidism. BAP guidelines (2016) recommend that if rapid cycling poses particular long-term management problems:

- **Identify and treat conditions such as hypothyroidism** or substance use that may contribute to cycling
- Consider **tapering and discontinuing antidepressants** that may contribute to cycling
- **Track mood states** longitudinally and **discontinue ineffective treatments** to avoid unnecessary polypharmacy

Ref: Goodwin GM et al. Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2016 Jun;30(6):495-553.

The correct answer is: Thyroid function test



Question 39

Not answered

Marked out of 1.00

A known bipolar patient has had many episodes of mania and depression. He has also experienced episodes of manic switch following SSRI treatment. Currently he presents with severe depression. He is already on lithium with good serum levels and is compliant. Of the options listed, what is the next best step in his management?

Select one:

- ☐ Consider MAOIs
- ☐ Consider haloperidol
- ☐ Consider quetiapine
- ☐ Consider ECT
- ☐ Consider tricyclics

Your answer is incorrect.

Explanation:

As reflected in the most recent NICE, BAP, and CANMAT/ISBD bipolar treatment guidelines, there are several options in this scenario, including the addition of olanzapine+fluoxetine, quetiapine, olanzapine, lamotrigine, or lurasidone.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

Goodwin GM et al. Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2016 Jun;30(6):495-553.

The correct answer is: Consider quetiapine



Question 40

Not answered

Marked out of 1.00

Which of the following is the most important predictor of relapse in a patient with bipolar disorder?

Select one:

- ☐ Family history of bipolar disorder
- ☐ Sex of the patient
- ☐ Age of the patient
- ☐ Presence of residual symptoms
- ☐ Dose needed for mood stabilisation

Your answer is incorrect.

Explanation:

Risk of recurrence after a bipolar mood episode is especially high (50% in one year and 75% at four years, and afterwards, 10% per year) compared with other psychiatric disorders. Time to relapse is **3X earlier** with **residual symptoms** of mania or depression affecting function after recovery from a mood episode compared to those who make a full recovery. In contrast, the rate of relapse in those who make a full recovery from index episode and have not relapsed in 4 years is about 10% per year.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

The correct answer is: Presence of residual symptoms

Question **41**

Not answered

Marked out of 1.00

A 35 year old woman is going to be discharged from hospital after successful treatment of a depressive episode. She has had 5 depressive episodes in the past and this time following depression she developed hypomania. What would you prescribe?

Select one:

- ☐ Carbamazepine
- ☐ Lamotrigine
- ☐ Lithium
- ☐ Atypical antipsychotic
- ☐ Sodium valproate

Your answer is incorrect.

Explanation:

NICE guidelines emphasize that lithium should be offered, as lithium is the most effective long-term treatment for bipolar disorder. NICE guidelines also state that sodium valproate must not be used in pregnancy and only used in girls and women when there is no alternative and a pregnancy prevention plan is in place.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

The correct answer is: Lithium



Question **42**

Not answered

Marked out of 1.00

In comparing bipolar depression with unipolar depression all are true except:

Select one:

- ☐ A family history of bipolar disorder is more common in bipolar depression
- ☐ There are some notable phenomenological differences
- ☐ Antidepressant induced hypomanic switch suggests bipolar disorder
- ☐ Psychotic depression in earlier adulthood is suggestive of bipolar disorder
- ☐ Atypical symptoms are more common in unipolar depression

Your answer is incorrect.

Explanation:



Features of a Depressive Episode More Predictive of Bipolar Disorder (not Unipolar Depression)	
Early age at onset	
Psychotic depression before age 25	
Postpartum depression, especially with psychotic features	
Rapid onset and offset of depressive episodes of short duration (<3 mos) or cyclothymia	
Depression with marked psychomotor retardation	
Atypical features (reverse vegetative signs)	
Bipolar family history or high-density, 3-generation pedigrees	
Hyperthymic temperament	
Hypomania associated with antidepressants	
Repeated (at least 3 times) loss of effectiveness of antidepressants after initial response	
Depressive mixed state (with psychomotor excitement, irritable hostility, racing thoughts)	

Adapted from Kaplan and Sadock's Synopsis of Psychiatry, 12th ed. 2022. p. 385.

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Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 385.

The correct answer is: Atypical symptoms are more common in unipolar depression

Question **43**

Not answered

Marked out of 1.00

Median time to recover from a treated episode of mania is

Select one:

- ☐ 2 - 3 weeks
- ☐ 1 year
- ☐ 4 - 5 weeks
- ☐ 12 - 18 weeks
- ☐ 6 months

Your answer is incorrect.

Explanation:

Bipolar disorder is usually chronic and recurrent. The average number of episodes is 10 (though there is large variation between patients). Episodes of mania and depression tend to cluster together. The median time to recover from mania (treated) is around 4-5 weeks.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

The correct answer is: 4 - 5 weeks

Question **44**

Not answered

Marked out of 1.00

A 24-year-old man has been using amphetamines for the last six months. He had an episode of depression when he was 22; this was treated by his GP using fluoxetine. He now presents with euphoria, increased energy & hyperactivity, and psychotic features suggestive of mania. He is admitted to hospital and remains drug-free for six weeks. However, his symptoms continue without change. The most appropriate diagnosis is

Select one:

- ☐ Mood disorder, unspecified
- ☐ Bipolar disorder
- ☐ Stimulant-induced psychosis
- ☐ Personality disorder
- ☐ Malingering

Your answer is incorrect.

Explanation:

Mania-like symptoms can be the result of using stimulant drugs such as cocaine, khat, ecstasy, or amphetamine. Typically, symptoms dissipate within 7 days after the substance is withdrawn, whereas primary manic symptoms last much longer. In stimulant or other street drug-induced mania, if the mood state significantly outlasts the drugged state, then a diagnosis of bipolar disorder may be made.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the assessment and management of bipolar disorder in adults, children and young people in primary and secondary care. Updated edition 2020.

The correct answer is: Bipolar disorder

Question 45

Not answered

Marked out of 1.00

Which of the following prodromal symptoms is more suggestive of schizophrenia rather than bipolar disorder?

Select one:

- ☐ Unexplained suicidality
- ☐ Unusual ideas
- ☐ Obsessions and compulsions
- ☐ Lack of energy
- ☐ Increased energy

Your answer is incorrect.

Explanation:

A predominantly insidious prodrome (>1 year) is seen in more than half of bipolar patients.

The period between first symptoms and diagnosis tends to be characterised by many months of gradual build-up of intensity and duration of subsyndromal symptoms such as depression, irritability, and switching from depression to brief periods of manic symptoms. For bipolar disorder and schizophrenia, prodromal characteristics can overlap considerably. **Unusual ideas** are significantly more likely to be part of a **schizophrenia prodrome**. Obsessions/compulsions, suicidality, difficulty thinking/communicating clearly, depressed mood, decreased concentration/memory, tiredness/lack of energy, mood lability, and physical agitation are more likely part of the bipolar prodrome.

Ref: Correll CU et al. Differentiation in the preonset phases of schizophrenia and mood disorders: evidence in support of a bipolar mania prodrome. Schizophrenia bulletin. 2007 May 1;33(3):703-14.

The correct answer is: Unusual ideas

Question **46**

Not answered

Marked out of 1.00

A 36 year old man treated for bipolar disorder wants to know which drug can reduce the risk of suicide in bipolar patients. The best option is

Select one:

- ☐ Lithium
- ☐ Lamotrigine
- ☐ Olanzapine
- ☐ Quetiapine
- ☐ Carbamazepine

Your answer is incorrect.

Explanation:

BAP guidelines for the treatment of bipolar disorder (2016) note that lithium is associated with a reduced risk of suicide in patients with bipolar disorder in RCTs, and in both self-harm and suicide in observational studies.

Ref: Goodwin GM et al. Evidence-based guidelines for treating bipolar disorder: revised third edition recommendations from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2016 Jun;30(6):495-553.

The correct answer is: Lithium



Question **47**

Not answered

Marked out of 1.00

Which of the following drugs is most likely to cause hyponatraemia?

Select one:

- ☐ Valproate
- ☐ Olanzapine
- ☐ Paroxetine
- ☐ Carbamazepine
- ☐ Risperidone

Your answer is incorrect.

Explanation:

All of the medications listed can cause hyponatremia, though it is probably more likely to occur in association with SSRIs, including citalopram, and anticonvulsants, particularly carbamazepine and oxcarbazepine. The estimates of incidence vary as per definitions used with 0.06 to 40% in some studies for SSRIs. Among SSRIs, fluoxetine, citalopram and escitalopram appear to have higher incidence rates (.078% to .085%) than paroxetine and sertraline (.033% to .053%). The incidence rates of SIADH varies between 4.8% and 41.5% for mood stabilizers, of which carbamazepine appears to have the highest incidence. Pharmacoepidemiological studies indicate that both citalopram and carbamazepine are common offenders.

Ref: Butterfield DJ, Eaves S, Ott C. Psychotropic-induced hyponatremia. Current Psychiatry. 2019 Feb 1;18(2):36-42.

Shepselovich D, et al. Medication-induced SIADH: distribution and characterization according to medication class. British journal of clinical pharmacology. 2017 Aug;83(8):1801-7.

The correct answer is: Carbamazepine

Question 48

Not answered

Marked out of 1.00

Which of the following statements about the diagnostic stability of bipolar disorder is true?

Select one:

- ☐ The diagnosis of bipolar disorder is often delayed in those with an index depressive phase
- ☐ More patients lose than retain a diagnosis of bipolar disorder
- ☐ Bipolar I to bipolar II switch happens frequently
- ☐ Bipolar I diagnosis is less stable than bipolar II
- ☐ After a 5-year symptom-free period, a diagnosis of bipolar can be removed

Your answer is incorrect.

Explanation

The initial diagnosis of bipolar disorder (most commonly a depressive episode) is often problematic; diagnostic delays of 8-10 years are common. Incorrect diagnoses in patients with bipolar disorder have been reported in 20-60% of cases. Once a diagnosis is made, the stability of bipolar disorder has generally been reported to be high, ranging from 60% to 80%. Bipolar I is more stable than bipolar II simply because bipolar II may switch to bipolar I, but not vice versa. Extreme ages, female sex, psychotic symptoms, changes to treatment, substance abuse, and family history of affective disorder have been related to diagnostic instability.

Ref: Cegla-Schwartzman FB et al. Diagnostic Stability in Bipolar Disorder: A Narrative Review. Harvard review of psychiatry. 2019 Jan 1;27(1):3-14.

The correct answer is: The diagnosis of bipolar disorder is often delayed in those with an index depressive phase

Question **49**

Not answered

Marked out of 1.00

Which of the following antidepressants is most likely to induce a mood switch from depression to mania?

Select one:

- ☐ Sertraline
- ☐ Paroxetine
- ☐ Fluoxetine
- ☐ Citalopram
- ☐ Amitriptyline

Your answer is incorrect.

Explanation:

Antidepressants can cause a switch to mania in those with or predisposed to bipolar disorder. Switching usually takes place soon (within 2 months) after starting antidepressants. Antidepressant-induced mania/hypomania has been reported with all major antidepressant classes in a subgroup of about 20-40% of bipolar patients.



Risk factors for treatment-emergent affective switch	
Bipolar I (versus Bipolar II)	
Mixed depressive features	
Rapid cycling	
Antidepressant subtype:	
TCAs > venlafaxine (?other SNRIs) > SSRIs or bupropion	
Substance abuse history, especially stimulants	

Adapted from Gitlin 2018



Ref: Gitlin MJ. Antidepressants in bipolar depression: an enduring controversy. International journal of bipolar disorders. 2018 Dec 1;6(1):25.

The correct answer is: Amitriptyline

Question 50

Not answered

Marked out of 1.00

All of the following are contraindications for zopiclone except

Select one:

- ☐ Sleep apnoea
- ☐ Pregnancy / breast feeding
- ☐ Unstable myasthenia gravis
- ☐ Respiratory depression
- ☐ Memory impairment

Your answer is incorrect.

Explanation:

Zopiclone is contraindicated in patients with myasthenia gravis, respiratory failure, sleep apnoea, and severe hepatic insufficiency. It has the capacity to depress respiratory drive. Use in pregnancy is not recommended. Zopiclone is excreted in breast milk and, although its concentration in breast milk is low, use in nursing mothers must be avoided. Anterograde amnesia may occur with zopiclone, but memory impairment is not a contraindication.

Ref: <https://www.medicines.org.uk/emc/product/4060/smpc#gref>

The correct answer is: Memory impairment

Question **51**

Not answered

Marked out of 1.00

Which of the following is frequently co-morbid with generalised anxiety disorder (GAD) and shares substantial overlap in diagnostic criteria with GAD?

Select one:

- ☐ Obsessive-compulsive disorder
- ☐ Major depressive disorder
- ☐ Schizophrenia
- ☐ Bipolar disorder
- ☐ Borderline personality disorder

Your answer is incorrect.

Explanation:

Generalised anxiety and depression are frequently comorbid: about 62% of patients with generalised anxiety disorder also have at least one episode of major depressive disorder during their lifetime. This finding is probably related to both a predisposition to mental illness and the substantial overlap in the diagnostic criteria for these disorders. Diminished ability to think or concentrate, restlessness, and sleep disturbance appear in the diagnostic criteria for both generalised anxiety and major depression.

Ref: Slee A et al. Trends in generalised anxiety disorders and symptoms in primary care: UK population-based cohort study. The British Journal of Psychiatry. 2021 Mar;218(3):158-64.

The correct answer is: Major depressive disorder

Question **52**

Not answered

Marked out of 1.00

With regard to PTSD, which of the following is true?

Select one:

- ☐ Chronic pain is associated with PTSD
- ☐ Resilience is not a dynamic quality
- ☐ The prevalence of PTSD is the same all over the world
- ☐ PTSD is overdiagnosed
- ☐ Prior trauma does not increase susceptibility to PTSD

Your answer is incorrect.

Explanation:

The association of chronic pain with PTSD has been well-demonstrated in the literature. Chronic pain is present in 20-80% of trauma cases, and PTSD is present in 10-50% of chronic pain cases.

Ref: Baldwin DS et al. Evidence-based pharmacological treatment of anxiety disorders, post-traumatic stress disorder and obsessive-compulsive disorder: a revision of the 2005 guidelines from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2014 May;28(5):403-39.

Bryant RA. Post-traumatic stress disorder: a state-of-the-art review of evidence and challenges. World psychiatry. 2019 Oct;18(3):259-69.

Brennstuhl MJ et al. Chronic pain and PTSD: Evolving views on their comorbidity. Perspectives in psychiatric care. 2015 Oct.

The correct answer is: Chronic pain is associated with PTSD

Question **53**

Not answered

Marked out of 1.00

With respect to the first-line medication management of PTSD in adults, which of the following are endorsed by NICE guidelines?

Select one:

- ☐ Amitriptyline and phenelzine
- ☐ Sertraline and paroxetine
- ☐ Mirtazapine and paroxetine
- ☐ Amitriptyline and fluoxetine
- ☐ Mirtazapine and fluoxetine

Your answer is incorrect.

Explanation:

NICE recommends the use of an SSRI (sertraline, paroxetine, or fluoxetine) or venlafaxine as first-line medication for the management of PTSD. In the UK, only paroxetine and sertraline are licensed for the treatment of PTSD. Paroxetine is more likely to be associated with discontinuation symptoms.

Ref: National Institute for Health and Clinical Excellence. NICE guideline Post-traumatic stress disorder 2018.

Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. p. 428.

The correct answer is: Sertraline and paroxetine

Question 54

Not answered

Marked out of 1.00

Which of the following neuroimaging findings is most consistent with OCD?

Select one:

- ☐ Decreased metabolism in the orbitocingulate region
- ☐ Increased metabolism in the dorsal prefrontal cortex
- ☐ Increased metabolism in the orbitocingulate region
- ☐ Decreased metabolism in the amygdala
- ☐ Increased metabolism in the amygdala

Your answer is incorrect.

Explanation:

Many investigators have contributed to the hypothesis that OCD involves dysfunction in a neuronal loop running from the orbital frontal cortex to the cingulate gyrus, striatum (caudate nucleus and putamen), globus pallidus, thalamus and back to the frontal cortex. Organic insult to these regions can produce obsessive and compulsive symptoms. The results of neurosurgical treatment of OCD strongly support this hypothesis. Surgical interruption of this loop by means of cingulotomy, anterior capsulotomy or subcaudate tractotomy brings about symptomatic improvement in a large proportion of patients unresponsive to all other treatments. FMRI shows increased orbitocingulate metabolism in OCD.

Ref: Moreira PS et al. The neural correlates of obsessive-compulsive disorder: a multimodal perspective. Translational psychiatry. 2017 Aug;7(8):e1224-.

The correct answer is: Increased metabolism in the orbitocingulate region

Question 55

Not answered

Marked out of 1.00

A 19-year-old man has been placed on leave from college after showing up drunk to give a presentation. He is spending a lot of time alone, as his friends have stopped calling due to his repeated refusals to go out with them. He doesn't want to join his family for his father's birthday, as he says he would be so tense that he may vomit at dinner. Which of the following is the most likely diagnosis?

Select one:

- ☐ Generalised anxiety disorder
- ☐ Claustrophobia
- ☐ Social anxiety disorder
- ☐ Adjustment disorder
- ☐ Depressive episode

Your answer is incorrect.

Explanation:

Social anxiety disorder is characterised by marked and excessive fear or anxiety that consistently occurs in one or more social situations such as social interactions (e.g. having a conversation), doing something while feeling observed (e.g. eating or drinking in the presence of others), or performing in front of others (e.g. giving a speech). The individual is concerned that they will act in a way, or show anxiety symptoms, that will be negatively evaluated by others. Relevant social situations are consistently avoided or else endured with intense fear or anxiety.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Social anxiety disorder

Question 56

Not answered

Marked out of 1.00

In PANDAS which of the following tests is most likely to be positive?

Select one:

- ☐ Anti-basal ganglia antibodies
- ☐ Anti DNase B
- ☐ pANCA
- ☐ Anti nuclear antibodies
- ☐ cANCA

Your answer is incorrect.

Explanation:

Elevated blood titres for streptococcal antibodies (ASO and Anti-DNase B) indicate prior streptococcal infection. However, of note, they do not provide information about the time or duration of that infection. These antibodies remain elevated for greater than 12 months in over 50% of children. Therefore, a single antibody titre elevation does not link a streptococcal infection to a tic or OCD event. There is little evidence to support these tests in children with OCD or tics. PANDAS remains a clinical diagnosis.

Ref: Gilbert DL et al. A pediatric neurology perspective on pediatric autoimmune neuropsychiatric disorder associated with streptococcal infection and pediatric acute-onset neuropsychiatric syndrome. The Journal of pediatrics. 2018 Aug 1;199:243-51.

The correct answer is: Anti DNase B

Question 57

Not answered

Marked out of 1.00

In PTSD there is confirmed effectiveness for all of the following treatments except

Select one:

- ☐ An antidepressant
- ☐ EMDR
- ☐ Hypnotherapy
- ☐ Imagery rescripting therapy
- ☐ Trauma-focused CBT

Your answer is incorrect.

Explanation:

The treatment of choice for PTSD is trauma-focused CBT. Trauma-focused CBT variants include prolonged exposure, cognitive therapy, cognitive processing therapy, and imagery rescripting therapy. All trauma-focused CBT treatments typically involve exposure, and comprise emotional processing of the traumatic memory and integration of new corrective information. Eye movement desensitisation and reprocessing (EMDR) for PTSD usually consists of 8-12 sessions of repeated bilateral stimulation (with eye movements) specifically targeting traumatic memories until memories are no longer distressing; this is recommended for non-combat-related trauma only. Meta-analyses have demonstrated that trauma-focused CBT and EMDR are both efficacious and superior to stress management and have similar overall efficacy. First-line pharmacological treatment for PTSD is an SSRI (paroxetine, sertraline, or fluoxetine) or venlafaxine. Hypnotherapy is not indicated and has no evidence base to support its use in PTSD.

Ref: National Institute for Health and Clinical Excellence. NICE guideline Post-traumatic stress disorder 2018.

Baldwin DS et al. Evidence-based pharmacological treatment of anxiety disorders, post-traumatic stress disorder and obsessive-compulsive disorder: a revision of the 2005 guidelines from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2014 May;28(5):403-39.

The correct answer is: Hypnotherapy

Question 58

Not answered

Marked out of 1.00

In the aetiology of PTSD choose one correct option:

Select one:

- ☐ locus of control has no role
- ☐ the sympathetic nervous system is not involved
- ☐ childhood trauma increases the risk
- ☐ the hippocampus has no role
- ☐ lower level of education is protective

Your answer is incorrect.

Explanation:

Neural changes in PTSD are consistent with circuitry implicated in fear conditioning, including the amygdala, prefrontal cortex, and hippocampus. Noradrenergic dysregulation may be key to the development of intrusive re-experiencing of traumatic memories (prazosin, a noradrenergic receptor inhibitor, helps reduce nightmares and re-experiencing symptoms of PTSD). Genetic factors account for 30-72% of the vulnerability to develop PTSD. However, genetic vulnerability to PTSD is moderated by contextual factors. Early life stress is particularly relevant, with evidence that childhood trauma modifies the genetic risk for PTSD. Risk factors for PTSD include female sex, low socio-demographic background, prior mental disorder, family history of mental disorders, and traumatic childhoods. Having less than a secondary level of education is also associated with a greater likelihood of developing PTSD.

Ref: Bryant RA. Post-traumatic stress disorder: a state-of-the-art review of evidence and challenges. World psychiatry. 2019 Oct;18(3):259-69.

The correct answer is: childhood trauma increases the risk

Question 59

Not answered

Marked out of 1.00

A 44 year old man with generalised anxiety disorder (GAD) wants to try an herbal remedy. Which of the following appears to have some beneficial effects in GAD but has been withdrawn in many countries due to potential hepatotoxicity?

Select one:

- ☐ Hypericum perforatum
- ☐ Gingko biloba
- ☐ Piper methysticum (Kava)
- ☐ Paxa ginseng
- ☐ Valerian officinalis

Your answer is incorrect.

Explanation:

The most recent BAP guidelines (2014) note that many patients with anxiety disorders wonder whether herbal preparations or nutritional supplements might be beneficial, either instead of or in conjunction with standard pharmacological or psychological treatments. Systematic reviews find some evidence for the potential effectiveness of a number of 'phytomedicines', including Passiflora species extracts, Kava (Piper methysticum), and combinations of L-lysine and L-arginine. There is no current convincing evidence for the effectiveness of homeopathic preparations in the treatment of patients with anxiety disorders. Kava preparations appeared to have some beneficial effects in patients with GAD but have been withdrawn in many countries due to potential hepatotoxic effects.

Ref: Baldwin DS et al. Evidence-based pharmacological treatment of anxiety disorders, post-traumatic stress disorder and obsessive-compulsive disorder: a revision of the 2005 guidelines from the British Association for Psychopharmacology. Journal of Psychopharmacology. 2014 May;28(5):403-39.

The correct answer is: Piper methysticum (Kava)

Question 60

Not answered

Marked out of 1.00

A 62-year-old woman is brought to hospital by her family for urgent assessment. Her children report that her home has become overrun with belongings that she has been storing for decades, many of which she has stolen and none of which she uses. She does not see the belongings as a problem and becomes very anxious when her children talk about throwing them away. She is uncooperative with assessment, presenting as anxious, distracted, and agitated. What is the most appropriate diagnosis?

Select one:

- ☐ Obsessive-compulsive disorder
- ☐ Kleptomania
- ☐ Dementia
- ☐ Hoarding disorder
- ☐ Attention deficit disorder

Your answer is incorrect.

Explanation:

Hoarding disorder is now recognised as a distinct disorder in DSM-5 (within the category of obsessive-compulsive and related disorders) and ICD-11. It was previously considered a subtype of OCD, though it is also seen in association with other disorders including anxiety disorders, ADHD, bipolar disorder, dementia, and schizophrenia. Hoarding disorder is characterised by persistent difficulty discarding or parting with possessions, regardless of their actual value. There is a strong perceived need to save the items and distress associated with discarding them. In 80-90% of cases, it is accompanied by excessive buying, collecting, and/or stealing items. Symptoms are 3X as common in older adults (age 55-94) than younger adults (age 34-44).

Ref: Piacentino D et al. Pharmacotherapy for Hoarding Disorder: How did the Picture Change since its Excision from OCD?. Current neuropharmacology. 2019 Aug 1;17(8):808-15.

Burki T. Hoarding disorder: a medical condition. The Lancet. 2018 Aug 25;392(10148):626.

The correct answer is: Hoarding disorder



Question **61**

Not answered

Marked out of 1.00

A previously well 24-year-old woman presents with nightmares, low mood, poor concentration, and feeling like she is in a daze. The symptoms started 3 weeks ago, after she was sexually assaulted by her roommate's friend. She is particularly distressed that she is having trouble remembering all the details of the rape and is worried that no one will believe her. What is the best diagnosis for her condition?

Select one:

- ☐ Post-traumatic stress disorder
- ☐ Acute stress reaction
- ☐ Acute stress disorder
- ☐ Adjustment disorder
- ☐ Derealisation disorder

Your answer is incorrect.

Explanation:

As defined in DSM-5, acute stress disorder (ASD) occurs in the first month after trauma exposure. It was initially introduced in DSM-IV as a means for describing severely distressed people who could not be diagnosed with PTSD in the initial month (because the duration of the disturbance in PTSD must be more than one month), and as a way to identify people who are high risk for later PTSD. Subsequent longitudinal studies have indicated that ASD is only a moderate predictor of PTSD: at least half of people who develop PTSD do not initially meet criteria for acute stress disorder. Initial conceptualisations of ASD placed much emphasis on dissociative responses immediately after trauma exposure (including depersonalisation, derealisation, reduced awareness of one's surroundings). However, many people who develop PTSD do not display dissociative responses in the acute phase after trauma, so this is no longer necessary for the ASD diagnosis.

Ref: Bryant RA. Post-traumatic stress disorder: a state-of-the-art review of evidence and challenges. World psychiatry. 2019 Oct;18(3):259-69.

International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Acute stress disorder

Question **62**

Not answered

Marked out of 1.00

The first line treatment in an adult with severe OCD is

Select one:

- ☐ Clomipramine
- ☐ CBT only
- ☐ Combined SSRI and CBT
- ☐ Antipsychotic
- ☐ SSRI only

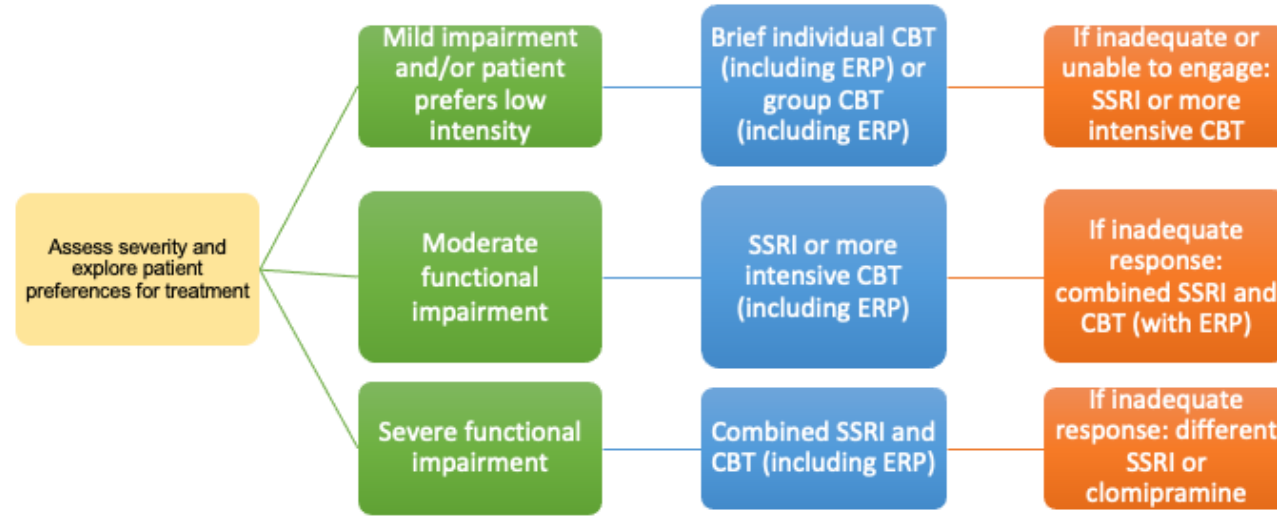
Your answer is incorrect.

Explanation:





Stepwise management in OCD



Adapted from Hirschtritt et al., JAMA. 2017; NICE Pathway, accessed 2020

Ref: National Institute for Health and Clinical Excellence. NICE guideline Obsessive-compulsive disorder: core interventions in the treatment of obsessive-compulsive disorder and body dysmorphic disorder. Updated edition 2020.

The correct answer is: Combined SSRI and CBT

Question 63

Not answered

Marked out of 1.00

Which of the following outcomes is often used to define treatment response in OCD treatment trials?

Select one:

- ☐ 35% reduction in YBOCS
- ☐ 50% reduction in YBOCS
- ☐ 75% reduction in YBOCS
- ☐ YBOCS scores fall below detectable levels
- ☐ 20% reduction in YBOCS

Your answer is incorrect.

Explanation:

Studies examining OCD treatment outcomes have typically used the Yale-Brown Obsessive Compulsive Scale (Y-BOCS). Reductions of 25-35% of the pre-treatment Y-BOCS score have been found to be the most reliably predictive of treatment response.

Ref: Farris SG, McLean CP, Van Meter PE, Simpson HB, Foa EB. Treatment response, symptom remission and wellness in obsessive-compulsive disorder. The Journal of clinical psychiatry. 2013 Jul;74(7):685.

National Institute for Health and Clinical Excellence. NICE guideline Obsessive-compulsive disorder: core interventions in the treatment of obsessive-compulsive disorder and body dysmorphic disorder. Updated edition 2020.

The correct answer is: 35% reduction in YBOCS

Question **64**

Not answered

Marked out of 1.00

Many patients with chronic fatigue syndrome will meet the criteria for

Select one:

- ☐ Generalised anxiety disorder
- ☐ Malingering
- ☐ Hypochondriasis
- ☐ Major depression
- ☐ OCD

Your answer is incorrect.

Explanation:

Many patients with chronic fatigue syndrome meet the criteria for other psychiatric diagnoses, most commonly major depression. Many resist a 'psychiatric' diagnosis, attributing mood disturbance to the restriction on activities caused by illness. Despite this, treatment of co-morbid depressive or anxiety symptoms can produce clinical improvement.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 875.

The correct answer is: Major depression

Question 65

Not answered

Marked out of 1.00

What percentage of patients with restless leg syndrome can present with periodic limb movements during sleep?

Select one:

- ☐ 10-20%
- ☐ 80-90%
- ☐ 1-2%
- ☐ 30-40%
- ☐ 50-70%

Your answer is incorrect.

Explanation:

Periodic limb movements of sleep are seen on sleep studies in 80% of patients with restless legs syndrome (RLS) and in more than one-fourth of the middle-aged European population. They are common in people both with and without RLS.

Ref: Trotti LM. Restless legs syndrome and sleep-related movement disorders. Continuum: Lifelong Learning in Neurology. 2017 Aug 1;23(4):1005-16.

The correct answer is: 80-90%



Question 66

Not answered

Marked out of 1.00

Which one of the following is not a diagnostic criterion for restless legs syndrome?

Select one:

- ☐ Symptoms worsen at night
- ☐ Symptoms improve with movement
- ☐ Symptoms worsen at rest
- ☐ Cataplexy
- ☐ Distressing urge to move the legs, usually accompanied by leg discomfort

Your answer is incorrect.

Explanation:

Classifications of restless legs syndrome vary slightly, but all contain five core criteria (mnemonic **URGED**):

- Urge to move the legs, often accompanied by leg discomfort
- Rest worsens the urge to move
- Getting up and moving improves the urge
- Evening or night worsens symptoms
- Disorders that mimic RLS have been excluded

Ref: Trotti LM. Restless legs syndrome and sleep-related movement disorders. Continuum: Lifelong Learning in Neurology. 2017 Aug 1;23(4):1005-16.

The correct answer is: Cataplexy

Question **67**

Not answered

Marked out of 1.00

Which of the following is a supportive criterion for restless legs syndrome?

Select one:

- ☐ Negative family history
- ☐ Worsening of symptoms in the evening
- ☐ Non-response to dopaminergic therapy
- ☐ Improvement of symptoms with movement
- ☐ Periodic limb movements of sleep

Your answer is incorrect.

Explanation:

Several supportive criteria for restless legs syndrome (RLS), which are useful in cases with equivocal symptom description, include periodic limb movements of sleep, first-degree relatives with RLS, and symptom improvement with a dopaminergic medication. Worsening of symptoms in the evening and improvement of symptoms with movement are core criteria, not supportive criteria.

Ref: Trotti LM. Restless legs syndrome and sleep-related movement disorders. Continuum: Lifelong Learning in Neurology. 2017 Aug 1;23(4):1005-16.

The correct answer is: Periodic limb movements of sleep

Question 68

Not answered

Marked out of 1.00

In REM sleep behaviour disorder, the associated lesions are situated in which part of the brain?

Select one:

- ☐ Cortex
- ☐ Brainstem
- ☐ Hippocampus
- ☐ Basal ganglia
- ☐ Uncus

Your answer is incorrect.

Explanation:

Normally REM sleep is associated with loss of muscle tone and dreaming. In REM sleep behaviour disorder (RBD), there is no loss of muscle tone, and dreams are acted out as complex behaviours with limited awareness of surroundings. RBD may occur idiopathically or may be associated with disorders such as Parkinson disease, Lewy body disease, multiple system atrophy, and Gullian-Barré syndrome. Associated lesions are likely situated in the brainstem.

Ref: Bassetti CL, Bargiotas P. REM sleep behavior disorder. In Neurologic-Psychiatric Syndromes in Focus-Part I 2018 (Vol. 41, pp. 104-116). Karger Publishers.

The correct answer is: Brainstem

Question 69

Not answered

Marked out of 1.00

First-line pharmacological treatment of restless legs syndrome is

Select one:

- ☐ Dopamine agonists
- ☐ Opioids
- ☐ Clonazepam
- ☐ Diazepam
- ☐ Anticonvulsants

Your answer is incorrect.

Explanation:

First line treatments of restless legs syndrome (RLS) include $\alpha 2\delta$ calcium channel ligands (gabapentin, pregabalin, and gabapentin enacarbil) and dopamine agonists (pramipexole, ropinirole, rotigotine). Prior to the advent of dopamine agonists, benzodiazepines were the treatment of RLS symptoms for a long time. Some cases were tried on baclofen and antiepileptic drugs (carbamazepine, valproic acid) with some success.

Ref: Trotti LM. Restless legs syndrome and sleep-related movement disorders. Continuum: Lifelong Learning in Neurology. 2017 Aug 1;23(4):1005-16.

Bollu PC et al. Sleep medicine: Restless legs syndrome. Missouri medicine. 2018 Jul;115(4):380.

The correct answer is: Dopamine agonists

Question 70

Not answered

Marked out of 1.00

Which one among the following is a maintaining factor for chronic fatigue syndrome (CFS)?

Select one:

- ☐ Transient inactivity after mononucleosis
- ☐ Pre-morbid mood disorder
- ☐ Neuroticism and introversion
- ☐ Serious life events
- ☐ Attributing the illness to a physical cause

Your answer is incorrect.

Explanation:

Perpetuating/maintaining factors in chronic fatigue syndrome (CFS):

- Attribution of CFS to a physical cause
- Pervasive activity avoidance
- Low sense of control
- Primary/secondary gains (e.g. sickness/disability benefit)
- Low self-perception of cognitive ability

Ref: Prochalska C et al. Chronic fatigue syndrome: An update for psychiatrists. Open Journal of Psychiatry 2012 2:40-8.

Cortes Rivera M et al. Myalgic encephalomyelitis/chronic fatigue syndrome: a comprehensive review. Diagnostics. 2019 Sep;9(3):91.

The correct answer is: Attributing the illness to a physical cause

Question 71

Not answered

Marked out of 1.00

Which of the following symptoms has the highest sensitivity for a diagnosis of obstructive sleep apnoea?

Select one:

- ☐ Choking during sleep
- ☐ Gasping during sleep
- ☐ Snoring
- ☐ Early morning headaches
- ☐ Excessive daytime sleepiness

Your answer is incorrect.

Explanation:

The most common symptoms associated with OSA include snoring that is bothersome to others, nocturnal awakening, nocturia, unrefreshing sleep, and daytime sleepiness resulting in reduced quality of life. Of the OSA symptoms, snoring has the highest sensitivity for OSA but is nonspecific. Patients with OSA are more likely than simple snorers to report loud, nightly snoring that is bothersome to others. Excessive daytime sleepiness is also a nonspecific symptom but if present, it influences therapy options and requires monitoring in response to therapy. Although relatively insensitive, choking or gasping during sleep is highly specific for moderate to severe OSA, as is the presence of early morning headaches.

Ref: Patel SR. Obstructive Sleep Apnea. Ann Intern Med. 2019 Dec 3;171(11):ITC81-ITC96.

The correct answer is: Snoring

Question 72

Not answered

Marked out of 1.00

Mr. X suffers from recurrent episodes of abrupt awakening from sleep characterized by a panicky scream, with intense fear. He has no recall of the details of the events and is unresponsive during the episodes. His most likely diagnosis is

Select one:

- ☐ Nightmares
- ☐ Sleepwalking
- ☐ Confusion arousals
- ☐ Sleep terrors
- ☐ REM sleep behaviour disorder

Your answer is incorrect.

Explanation:

Sleep terrors are recurrent episodes of abrupt awakening from sleep characterised by a panicky scream, with intense fear and autonomic arousal. The individual usually has no recall of the details of the event and is unresponsive to stimuli (e.g conversation) during the episode. Like sleepwalking, these episodes usually arise from slow-wave sleep. Unlike nightmares, in which an elaborate dream sequence unfolds, sleep terrors may be devoid of images or contain only fragments of very brief but frighteningly vivid and sometimes static images. Psychopathology is seldom associated with sleep terrors in children; however, a history of a traumatic experience or frank psychiatric problems is often co-morbid in adults with this disorder.

Ref: Boland R, Verduin M. Kaplan and Sadock's Synopsis of Psychiatry, 12th Edition. 2022. p. 499.

The correct answer is: Sleep terrors



Question **73**

Not answered

Marked out of 1.00

Initial insomnia is frequently related to which of the following conditions?

Select one:

- ☐ Enuresis
- ☐ Pain syndromes
- ☐ Medical illness
- ☐ Anxiety disorders
- ☐ Depression

Your answer is incorrect.

Explanation:

Initial insomnia is characterized by difficulty falling asleep, with increased sleep latency. It is frequently related to anxiety disorders. In anxiety disorders, the pattern of sleep disturbances are very similar to primary insomnia, with long latency (initial insomnia) but no notable increase in awakenings.

Middle insomnia refers to difficulty maintaining sleep, often with frequent awakenings. It is often non-specific and is associated with medical illness, pain syndromes, or depression. In chronic pain syndromes, patients often exhibit disturbances in all 3 phases of sleep (initial, middle and late), and also have co-morbid depression and anxiety. The severity of insomnia tracks severity of pain and disability in many cases.

In terminal insomnia or early morning waking, patients consistently wake up earlier than needed. This is frequently associated with major depression.

Ref: Sun Q, Tan L. Comparing primary insomnia to the insomnia occurring in major depression and general anxiety disorder. Psychiatry Research. 2019 Dec 1;282:112514.

Sun Y, Laksono I, Selvanathan J, Saripella A, Nagappa M, Pham C, Englesakis M, Peng P, Morin CM, Chung F. Prevalence of sleep disturbances in patients with chronic non-cancer pain: A systematic review and meta-analysis. Sleep medicine reviews. 2021 Jun 1;57:101467.

The correct answer is: Anxiety disorders



Question **74**

Not answered

Marked out of 1.00

Which neuropeptide has been found to be low in patients with narcolepsy type 1?

Select one:

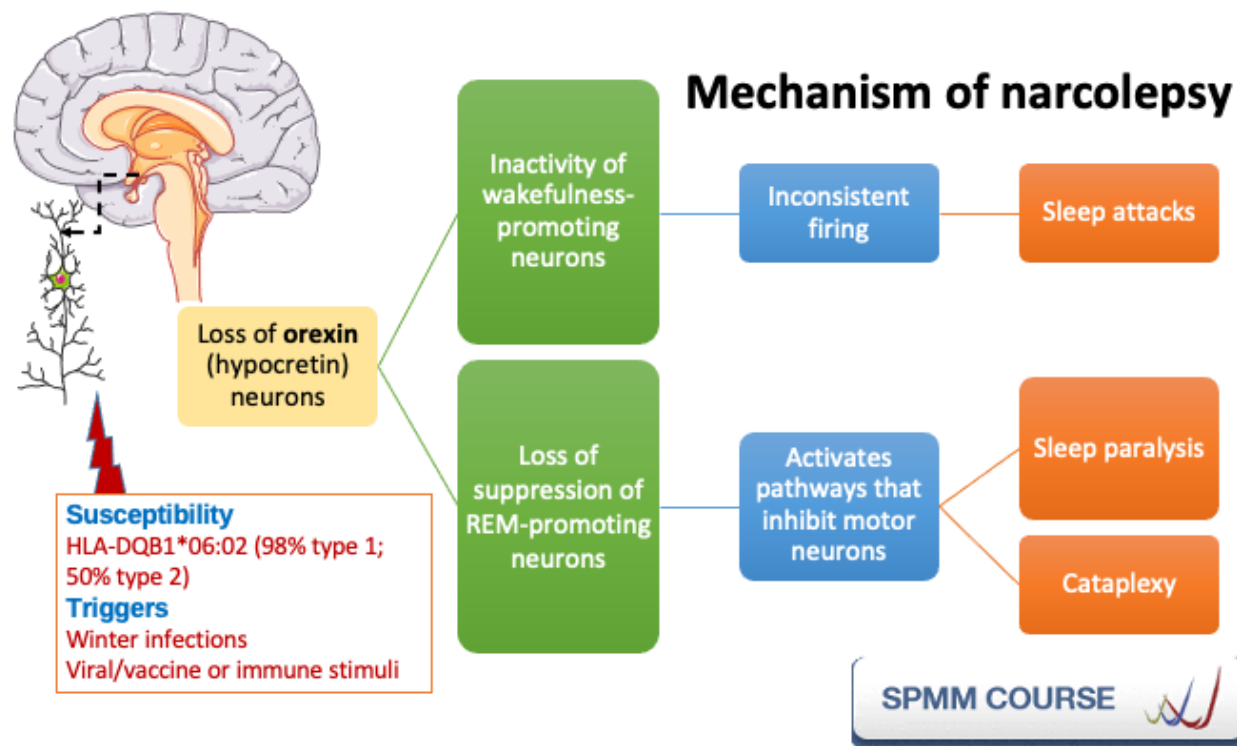
- ☐ Met/leu-enkephalins
- ☐ Beta-endorphins
- ☐ Hypocretin-1
- ☐ Neuropeptide-Y
- ☐ Neurotensin

Your answer is incorrect.

Explanation:

Hypocretins (orexins) are hypothalamic neuropeptides believed to play a role in regulating sleep and arousal. Abnormally low concentrations of hypocretin-1 (orexin-A) have been found in the CSF of patients with narcolepsy type 1 (with cataplexy).





Ref: Kornum BR et al. Narcolepsy. Nature reviews Disease primers. 2017 Feb 9;3(1):1-9.

The correct answer is: Hypocretin-1

Question 75

Not answered

Marked out of 1.00

Which of the following is a good prognostic factor in chronic fatigue syndrome (CFS)?

Select one:

- ☐ Older age
- ☐ Longer illness duration
- ☐ Not attributing CFS to a physical cause
- ☐ Severe fatigue
- ☐ Belief in complete rest as the treatment

Your answer is incorrect.

Explanation:

Good prognostic factors in chronic fatigue syndrome (CFS):

- Lack of attribution of CFS to a physical cause
- Lower severity of initial symptoms, including fatigue
- Engagement in regular physical activity
- High sense of control
- Younger age (children and adolescents)

Ref: Prochalska C et al. Chronic fatigue syndrome: An update for psychiatrists. Open Journal of Psychiatry 2012 2:40-8.

Cortes Rivera M et al. Myalgic encephalomyelitis/chronic fatigue syndrome: a comprehensive review. Diagnostics. 2019 Sep;9(3):91.

The correct answer is: Not attributing CFS to a physical cause

Question **76**

Not answered

Marked out of 1.00

Which immunological test is diagnostic of chronic fatigue syndrome?

Select one:

- ☐ Deficient interleukin 6
- ☐ Increased expression of activation markers on the surface of T lymphocytes
- ☐ Deficiency in natural killer cell function
- ☐ Increased number of CD8+ cytotoxic T cells bearing antigenic marker
- ☐ None of the listed options

Your answer is incorrect.

Explanation:

There are no diagnostic tests for chronic fatigue syndrome (CFS). The diagnosis involves unexplained fatigue for at least six months, at least four out of eight minor symptoms, and the absence of other fatigue-associated conditions. There is compelling evidence that all the immunological alterations seen in the pathophysiology of CFS are associated with autoimmunity. The search for autoantibodies is of great importance, allowing the development of potential biomarkers for the diagnosis. However, this is not available at present.

Ref: Cortes Rivera M et al. Myalgic encephalomyelitis/chronic fatigue syndrome: a comprehensive review. *Diagnostics*. 2019 Sep;9(3):91.

The correct answer is: None of the listed options

Question **77**

Not answered

Marked out of 1.00

Which of the following is classified as a non-REM (NREM)-related parasomnia?

Select one:

- ☐ Nightmare disorder
- ☐ Sleep enuresis
- ☐ Sleepwalking
- ☐ Narcolepsy
- ☐ Exploding head syndrome

Your answer is incorrect.

Explanation:

Parasomnias may be defined as undesirable physical and/or experimental phenomena accompanying sleep. They include unusual behaviours and motor acts, autonomic changes, and/emotional-perceptual events. In ICSD-3, NREM-related parasomnias include disorders of arousal, confusional arousals, sleepwalking, sleep terrors, and sleep-related eating disorder. REM-related parasomnias include REM sleep behaviour disorder, recurrent isolated sleep paralysis, and nightmare disorder. Other parasomnias include exploding head syndrome, sleep-related (hypnic) hallucinations, and sleep enuresis.

Ref: Semple D, Smyth R. Oxford handbook of psychiatry. Oxford university press; 2019 Jul 30. p. 458.

The correct answer is: Sleepwalking

Question **78**

Not answered

Marked out of 1.00

Diagnostic features of chronic fatigue syndrome include all of the following except

Select one:

- ☐ Fatigue alleviated by prolonged rest
- ☐ Post-exertional malaise
- ☐ Poor (unrefreshing) sleep
- ☐ Duration greater than 6 months
- ☐ Muscular aches and pains

Your answer is incorrect.

Explanation:



Criteria	Definition of Chronic Fatigue Syndrome (US Center for Disease Control and Prevention, 1994)
Main criteria	Persistent or relapsing fatigue of 6 months duration or more Fatigue being not explained by any concurrent somatic or psychiatric condition Fatigue is new or has a defined onset Fatigue is not due to ongoing exertion Fatigue is not relieved by rest
Additional criteria (at least 4)	Impaired memory and/or concentration that causes a reduction in previous level of activity Sore throat Tender cervical and/or axillary lymph nodes Muscle pain Multi-joint pain New headaches Unrefreshing sleep Post-exertional malaise
Exclusion criteria	Medical condition explaining fatigue (e.g. untreated hypothyroidism, sleep apnoea, narcolepsy) Previously diagnosed medical condition whose resolution has not been documented Past or current diagnosis of MDD or bipolar disorder, schizophrenia, dementia, delusional disorder, anorexia nervosa, bulimia nervosa, substance use disorder within 2 years before onset Severe obesity (BMI > 45) Unexplained physical exam/lab/imaging abnormality that strongly suggests exclusionary condition



Ref: Cortes Rivera M et al. Myalgic encephalomyelitis/chronic fatigue syndrome: a comprehensive review. Diagnostics. 2019 Sep;9(3):91.
 The correct answer is: Fatigue alleviated by prolonged rest

Question 79

Not answered

Marked out of 1.00

The most common problem seen in patients with medically unexplained symptoms is the potential for

Select one:

- ☐ Parasuicide
- ☐ Violence
- ☐ Iatrogenic harm
- ☐ Suicide
- ☐ Hostility

Your answer is incorrect.

Explanation:

Patients with persistent medically unexplained symptoms are at risk of iatrogenic harm as they may go through numerous investigations and receive unnecessary treatment such as medication and surgery. These patients often accrue considerable morbidity, and even mortality, due to excess negative investigations, irradiation, operative procedures, etc. Those disorders associated with chronic pain carry the risk of iatrogenic opioid dependency. Often at later stages in the patient's illness, this secondary morbidity is more problematic than the original symptoms. A major positive intervention in these patients is therefore the avoidance of iatrogenic harm.

Ref: Rosendal M et al. "Medically unexplained" symptoms and symptom disorders in primary care: prognosis-based recognition and classification. BMC family practice. 2017 Dec;18(1):1-9.

The correct answer is: Iatrogenic harm

Question **80**

Not answered

Marked out of 1.00

Which of the following modes of psychological treatment is not endorsed by evidence-based guidelines for managing anorexia?

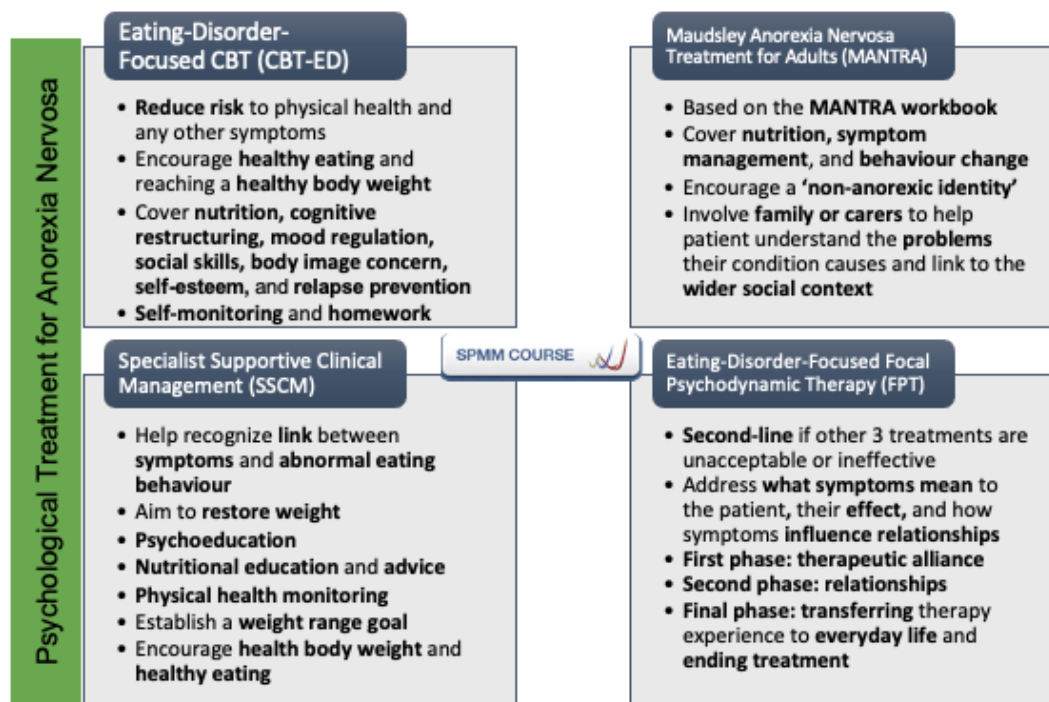
Select one:

- ☐ MANTRA
- ☐ Specialist supportive clinical management
- ☐ Cognitive behavioural therapy
- ☐ Focal psychodynamic therapy
- ☐ Dialectic behavioural therapy

Your answer is incorrect.

Explanation:





Ref: Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020;1-1.

National Institute for Health and Clinical Excellence. NICE guideline Eating Disorders: recognition and treatment 2017.

The correct answer is: Dialectic behavioural therapy

Question **81**

Not answered

Marked out of 1.00

Which of the following is a characteristic feature of borderline personality disorder?

Select one:

- ☐ Attention seeking behaviour
- ☐ Fear of abandonment
- ☐ Avoidance of social contact
- ☐ Generalised lack of trust
- ☐ Fear of boredom

Your answer is incorrect.

Explanation:

Features of borderline personality disorder (5/9 required for DSM-5 diagnosis):

- Unstable self-image
- Unstable personal relationships
- Impulsive behaviour in areas such as personal safety and substance misuse
- Self-harm, suicidal threats or gestures
- Affective instability due to marked reactivity of mood
- Episodes of micro-psychosis
- Chronic feelings of emptiness
- Frantic efforts to avoid abandonment
- Inappropriate, intense anger

Ref: American Psychiatric Association. Diagnostic and statistical manual of mental disorders (DSM-5®). American Psychiatric Pub; 2013 May 22.

The correct answer is: Fear of abandonment



Question 82

Not answered

Marked out of 1.00

A 28-year-old man is brought to the Accident & Emergency unit by the police after becoming violent with his girlfriend and then threatening suicide when she tried to leave. Upon arrival, he smells of alcohol and he admits to drinking excessively on a frequent basis. He has been arrested 8 times in the past for assaulting former girlfriends and has had multiple previous relationships. In his youth, he had a history of truancy, aggression towards peers, and getting in trouble with the police for possession of drugs. Your examination reveals no indication of a major mood, anxiety, or psychotic disorder. What would you do?

Select one:

- ☐ Refer him for psychodynamic psychotherapy
- ☐ Prescribe an antidepressant for his suicidality
- ☐ Prescribe an antipsychotic for his aggression
- ☐ Prescribe lithium for his impulsivity
- ☐ Refer him for treatment of his alcohol use disorder

Your answer is incorrect.

Explanation:

Those with antisocial personality disorder (ASPD) who misuse drugs or alcohol should be offered appropriate treatment of co-morbid substance use disorders. Be aware of the potential for high attrition, misuse of prescribed medication, and drug interactions (including with alcohol and illicit drugs). Consider offering group-based CBT to address impulsivity, interpersonal difficulties, and antisocial behaviour. Pharmacological interventions should NOT be routinely used for the treatment of ASPD or associated behaviours of aggression, anger, and impulsivity.

Ref: National Institute for Health and Clinical Excellence. The NICE guideline on the treatment, management, and prevention of antisocial personality disorder. Updated edition 2018.

The correct answer is: Refer him for treatment of his alcohol use disorder

Question **83**

Not answered

Marked out of 1.00

Which of the following is not commonly associated with bulimia?

Select one:

- ☐ Dental decay
- ☐ Parotid gland enlargement
- ☐ Oesophageal tears
- ☐ Peptic ulcer
- ☐ Seizures

Your answer is incorrect.

Explanation:

Repeated vomiting can result in oesophageal tears and dental erosion. Seizures may occur due to repeated vomiting and resulting electrolyte imbalance. Parotid enlargement is secondary to repeated vomiting and salivary secretion. Peptic ulcer is not directly related to bulimia.



Signs and Symptoms of Eating Disorders	
Skin, Hair, and Teeth	Dry skin; fine downy hair (lanugo) on the back, forearms, and side of the face; orange discolouration skin (hypercarotenaemia); alopecia; Russell's sign (calluses on knuckles due to repeated vomiting induction); dental erosion/decay; starvation-associated pruritis
Endocrine	Amenorrhoea (in females not taking an oral contraceptive); reduced sex drive; infertility; osteopenia and osteoporosis; thyroid abnormalities (low triiodothyronine – T3); hypercortisolaemia; hypoglycaemia
Cardiovascular	Bradycardia (around 40 is common); (orthostatic) hypotension; mitral valve prolapse; arrhythmias and sudden death; chest pain; dizziness and syncope
Gastrointestinal	Enlargement of parotid and submandibular glands (especially in bulimic patients); GI reflux; Barrett's esophagus; Mallory-Weiss (oesophageal) tear; constipation; refeeding pancreatitis; acute gastric dilatation; delayed gastric emptying (leading to pain and bloatedness); elevated liver enzymes
Haematological	Low white cell count (especially neutrophils); pancytopenia
Metabolic	Hypokalemia; dehydration; nephropathy; metabolic acidosis
Pulmonary	Respiratory failure; spontaneous pneumothorax
Neurological	Cerebral atrophy, seizures (due to electrolyte imbalance)

Adapted from Morris and Anderson 2020



Ref: Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020:1-11.

The correct answer is: Peptic ulcer

Question **84**

Not answered

Marked out of 1.00

Which of the following is a laboratory abnormality seen in anorexia nervosa?

Select one:

- ☐ High oestrogen
- ☐ Low cortisol
- ☐ Hyperkalaemia
- ☐ Low triiodothyronine
- ☐ Hyperphosphataemia

Your answer is incorrect.

Explanation:



Complications of Anorexia Nervosa		
Organ systems or organ	Pathological findings	Leading systems
CNS	Morphological and functional cerebral changes; volume reduction in cerebral grey and white matter	Cognitive deficits
Dental systems and parotid glands	Impaired dental status, dental caries, increased serum amylase	Dental caries, enlargement of the parotid glands
Endocrine system and reproductive function	Hypothalamus-pituitary-gonadal-axis, low T3 syndrome, hypercortisol	Amenorrhoea in women, symptoms of hypothyroidism, depression, elevated stress levels
Cardiovascular system	Hypotension, bradycardia, arrhythmia	Syncope
Gastrointestinal tract	Impaired gastric emptying, gastric dilation, gastro-duodenal ulcers	Constipation, ileus, upper gastrointestinal bleeding
Haematological and immune system	Bone marrow hypoplasia, anaemia with reduced leucocytes and immunoglobulin	Anaemia, bacterial infections, compromised immune competence
Renal tract	Hypokalemia, hypophosphataemia, hypernatraemia	Nephrolithiasis, oedema, syncope
Bone	Reduced bone density (osteopenia) or osteoporosis	Bone fractures and concomitant pain, spinal compression

Adapted from Zipfel et al. 2015



Ref: Zipfel S et al. Anorexia nervosa: aetiology, assessment, and treatment. The lancet psychiatry. 2015 Dec 1;2(12):1099-111.

Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020:1-11.

The correct answer is: Low triiodothyronine

Question 85

Not answered

Marked out of 1.00

Which of the following clusters of personality disorders is most commonly seen in psychiatric outpatient samples?

Select one:

- ☐ Cluster A
- ☐ Cluster C
- ☐ Cluster D
- ☐ Cluster E
- ☐ Cluster B

Your answer is incorrect.

Explanation:

In psychiatric OPD samples a distribution of the following pattern is noted: Cluster A = 6% Cluster B = 13% Cluster C = 22% Any PD = 46%.

Epidemiological studies on personality disorders in community samples are rare, and prevalence rates vary substantially depending on samples and methods. Prevalence rates are fairly high for any personality disorder (12%) and similarly high for DSM Clusters A, B, and C, between 5.5 and 7.3%. Prevalence has been shown to be highest for obsessive-compulsive personality disorder (4.3%) and lowest for dependent personality disorder (0.8%). DSM-5 rates are not yet available.

Ref: Zimmerman M, Rothschild L, Chelminski I. The prevalence of DSM-IV personality disorders in psychiatric outpatients. American Journal of Psychiatry. 2005 Oct 1;162(10):1911-8.

Volkert J et al. Prevalence of personality disorders in the general adult population in Western countries: systematic review and meta-analysis. The British Journal of Psychiatry. 2018 Dec;213(6):709-15.

The correct answer is: Cluster C

Question 86

Not answered

Marked out of 1.00

Which of the following is known to have a strong familial component, with estimates of heritability between 28 and 74%?

Select one:

- ☐ Bulimia nervosa
- ☐ Intermittent explosive disorder
- ☐ Anorexia nervosa
- ☐ Gambling disorder
- ☐ Binge eating disorder

Your answer is incorrect.

Explanation:

Early family studies, large twin-based studies and smaller adoption studies demonstrate both a heritable component AND significant influence of non-shared environmental factors in eating disorders. There is a strong familial component to anorexia nervosa, with estimates of heritability between 28 and 74%. The genetic contribution is polygenic. Large genome-wide association studies highlight the need to consider psychiatric and metabolic mechanisms involved in anorexia nervosa.

Ref: Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020:1-11.

The correct answer is: Anorexia nervosa

Question **87**

Not answered

Marked out of 1.00

A patient with borderline personality disorder exhibits emotional instability, transient quasi-psychotic symptoms, repeated self-harm, and impulsivity. Which of the following should be prescribed to this patient?

Select one:

- ☐ An antipsychotic
- ☐ An antidepressant
- ☐ A mood stabiliser
- ☐ A benzodiazepine
- ☐ None of the listed options

Your answer is incorrect.

Explanation:

Several aspects of borderline personality disorder (BPD) have been assumed to be responsive to drug treatment. These include affective instability, transient or stress-related quasi-psychotic or depressive symptoms, suicidal and self-harming behaviours, and impulsivity. However, no drug is specifically licensed for the treatment of BPD, or any aspect of BPD. Psychological treatments such as DBT are much better supported. NICE advises that drug treatment should not be used routinely for borderline personality disorder or for the individual symptoms or behaviour associated with the disorder (for example, repeated self-harm, marked emotional instability, risk-taking behaviour, and transient psychotic symptoms). Drug treatment may be considered in the overall treatment of co-morbid conditions.

Ref: Taylor DM, Barnes TRE, Young AH. The Maudsley Prescribing Guidelines in Psychiatry, 14th Edition. 2021. pp. 757-758.

The correct answer is: None of the listed options

Question **88**

Not answered

Marked out of 1.00

Which of the following are common physical features seen in anorexia nervosa?

Select one:

- ☐ Bradycardia and hypertension
- ☐ Normal pulse and blood pressure
- ☐ Bradycardia and hypotension
- ☐ Tachycardia and hypotension
- ☐ Tachycardia and hypertension

Your answer is incorrect.

Explanation:



Complications of Anorexia Nervosa		
Organ systems or organ	Pathological findings	Leading systems
CNS	Morphological and functional cerebral changes; volume reduction in cerebral grey and white matter	Cognitive deficits
Dental systems and parotid glands	Impaired dental status, dental caries, increased serum amylase	Dental caries, enlargement of the parotid glands
Endocrine system and reproductive function	Hypothalamus-pituitary-gonadal-axis, low T3 syndrome, hypercortisol	Amenorrhoea in women, symptoms of hypothyroidism, depression, elevated stress levels
Cardiovascular system	Hypotension, bradycardia, arrhythmia	Syncope
Gastrointestinal tract	Impaired gastric emptying, gastric dilation, gastro-duodenal ulcers	Constipation, ileus, upper gastrointestinal bleeding
Haematological and immune system	Bone marrow hypoplasia, anaemia with reduced leucocytes and immunoglobulin	Anaemia, bacterial infections, compromised immune competence
Renal tract	Hypokalemia, hypophosphataemia, hypernatraemia	Nephrolithiasis, oedema, syncope
Bone	Reduced bone density (osteopenia) or osteoporosis	Bone fractures and concomitant pain, spinal compression

Adapted from Zipfel et al. 2015



Ref: Zipfel S et al. Anorexia nervosa: aetiology, assessment, and treatment. The lancet psychiatry. 2015 Dec 1;2(12):1099-111.

Morris J, Anderson S. An update on eating disorders. BJPsych Advances. 2020:1-11.

The correct answer is: Bradycardia and hypotension

Question 89

Not answered

Marked out of 1.00

According to ICD-11, the core feature of a binge eating episode is

Select one:

- ☐ Eating until one is full
- ☐ Eating for at least 4 hours
- ☐ Loss of control over eating
- ☐ Eating in order to feel calm
- ☐ Pre-planning the eating session

Your answer is incorrect.

Explanation:

According to ICD-11, binge eating is defined as a discrete period of time (e.g., 2 hours) during which the individual experiences a loss of control over their eating behaviour and eats notably more or differently than usual. Loss of control over eating may be described by the individual as feeling like they cannot stop or limit the amount or type of food eaten; difficulty stopping eating once they have started; or giving up even trying to control their eating because they know they will end up overeating. Binge eating episodes may be 'objective', in which the individual eats an amount of food that is larger than most people would eat under similar circumstances, or 'subjective', which may involve eating amounts of food that might be objectively considered to be within normal limits but are subjectively experienced as large by the individual. In either case, the core feature of a binge eating episode is the experience of loss of control over eating. Binge eating is typically experienced as very distressing.

Ref: International Statistical Classification of Diseases and Related Health Problems (11th ed. ICD-11; World Health Organization, 2020)

The correct answer is: Loss of control over eating

Question 90

Not answered

Marked out of 1.00

A patient presents with impulsivity and anger outbursts that have led to several criminal charges. He is unapologetic for his actions and shows no empathy for the people he has harmed. Which of the following would be the most appropriate term to describe this patient?

Select one:

- ☐ Borderline
- ☐ Avoidant
- ☐ Narcissistic
- ☐ Obsessive
- ☐ Dissocial

Your answer is incorrect.

Explanation:

The term “dissocial”, as used in ICD, may be equated with “antisocial” as used in DSM classification. In accordance with ICD-11, dissociality is characterised by lack of empathy including callous, deceptive, manipulative, exploiting, mean, ruthless, and physically aggressive behaviour, and sometimes taking pleasure in inflicting pain or harm. Features of dissociality include impulsivity, irresponsibility, recklessness, and lack of planning without regard for risks or consequences. There is an absence of vulnerability, shame, and anxiety.

Ref: Bach B, First MB. Application of the ICD-11 classification of personality disorders. BMC psychiatry. 2018 Dec 1;18(1):351.

The correct answer is: Dissocial



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